GSSSB · STAFF NURSE EXAM

Mental Health Nursing (Psychiatry)

Complete Colourful Notes · 3D Diagrams · Mnemonics · Exam Points



12 Chapters | 3D Coloured Diagrams | MSE · Therapeutic communication | Psychiatric disorders | Psychopharmacology & ECT | Exam Points (PYQ style)

Hinglish explanation + English exam terms



Sister Asha
Aapki Nursing teacher

Index – Kya padhenge?

1 **Introduction, History & Concepts of Mental Health**
Mental health & illness definitions, history & pioneers, classification (ICD-11/ DSM-5), psychiatric terms (signs & symptoms), personality, Freud & theories, mental health team, psychiatric nurse roles

2 **Psychiatric Assessment, Therapeutic Communication & Nurse–Patient Relationship**
Psychiatric history, MSE, rating scales, therapeutic & non-therapeutic communication techniques, Peplau's phases, Johari window, transference, nursing process in psychiatry

3 **Psychopharmacology**
Antipsychotics (typical, atypical, depot), EPS & NMS, antidepressants (SSRI, TCA, MAOI), serotonin syndrome, mood stabilisers (lithium), anxiolytics & hypnotics, anticholinergics, drugs for dementia, ADHD, substance use; nursing responsibilities

4 **ECT, Psychotherapies & Therapeutic Modalities**
Electroconvulsive therapy (indications, procedure, nursing care), other somatic therapies, psychotherapies – CBT, behaviour therapy, psychoanalysis, group & family therapy, milieu therapy, OT, crisis intervention, restraint & seclusion

5 **Schizophrenia & Other Psychotic Disorders**
Aetiology, Bleuler's 4 A's, Schneider's first-rank symptoms, positive & negative symptoms, types, course & prognosis, treatment, nursing care of hallucinations, delusions, withdrawal; delusional disorder, acute psychosis, schizoaffective, postpartum psychosis

6 **Mood (Affective) Disorders & Suicide**
Depression (features, types, treatment, nursing care), mania & bipolar disorder (nursing care), dysthymia, cyclothymia, seasonal & postpartum depression, suicide – epidemiology, risk factors, warning signs, assessment, prevention & nursing management

7 **Anxiety, Obsessive-Compulsive, Stress-Related, Dissociative & Somatoform Disorders**
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8 **Substance Use Disorders**
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9 **Personality, Eating, Sleep & Sexual Disorders**
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10 **Neurocognitive (Organic) Disorders & Child-Adolescent Psychiatry**
Delirium vs dementia, Alzheimer's & other dementias, nursing care, MMSE; intellectual disability, ADHD, autism, conduct disorder, learning disorders, enuresis, tic disorders, child abuse; geriatric psychiatry

11 **Psychiatric Emergencies, Community Mental Health & Legal Aspects**
Violence & aggression management, catatonic stupor, NMS, serotonin syndrome, lithium toxicity, panic, drug overdose; community psychiatry, DMHP, rehabilitation; Mental Healthcare Act 2017 – admissions, rights, MHRB; forensic psychiatry; ethics

12 **Quick Revision – Master Tables & 150 One-Liners**
All psychiatry facts – pioneers, terms, drugs & doses, durations, scales, defence mechanisms, MHCA sections & 150 one-liners

★ **Key Points** – sabse important facts

Asha Didi ki Tip – samjhane wali baat

! Nursing Alert – galti mat karna

✎ **Exam Point** – PYQ style, baar baar aata hai

♫ **Mnemonic** – yaad karne ki trick

D Definition – exact definition

1 Introduction, History & Concepts of Mental Health

1.1 Definitions

D Mental health (WHO)

"A state of **well-being** in which an individual **realises his or her own abilities, can cope with the normal stresses of life, can work productively** and is able to **make a contribution to his or her community**." **Mental illness** – clinically significant disturbance in **cognition, emotion regulation or behaviour** causing **distress or impaired functioning**.

- **Characteristics of mentally healthy person** – positive self-concept, realistic perception, **adaptability/ coping**, autonomy, **self-actualisation (Maslow)**, environmental mastery, integration, good interpersonal relations.
- **Psychiatry** – branch of medicine dealing with mental disorders; **psychiatric (mental health) nursing** – specialised area using theories of human behaviour as its science & **purposeful use of self as its art** (ANA).
- **World Mental Health Day – 10 October**; **NIMHANS – Bengaluru**; **Central Institute of Psychiatry (CIP) – Ranchi (1918)**; **Hospital for Mental Health – Ahmedabad, Vadodara, Jamnagar, Bhuj (Gujarat)**.
- **Prevalence (National Mental Health Survey 2015–16, NIMHANS)** – **~10.6 % adults** have a mental disorder; **treatment gap 70–92 %**; common – **depression, anxiety, substance use (alcohol)**.

1.2 History & Pioneers

Person	Contribution
Hippocrates	Natural causes of mental illness (humours) – "melancholia"
Philippe Pinel	**Removed chains** from mentally ill (Bicêtre, Paris, **1793**) – **"moral treatment"**; often called **father of modern psychiatry**
William Tuke	York Retreat (England) – humane care
Benjamin Rush	**Father of American psychiatry**
Dorothea Dix	Reform of asylums (USA)
Emil Kraepelin	Classified mental illness; **"dementia praecox"** (schizophrenia) & manic-depressive psychosis
Eugen Bleuler	Coined **"schizophrenia" (1908/1911)**; **4 A's**
Kurt Schneider	**First-rank symptoms** of schizophrenia
Sigmund Freud	**Father of psychoanalysis**; id-ego-superego; defence mechanisms; free association, dream analysis
Linda Richards	**First American psychiatric nurse**; started first training school for psychiatric nurses (1882)
Hildegard Peplau	**Mother of psychiatric nursing**; **Interpersonal Relations in Nursing (1952)**
Cerletti & Bini	**ECT (1938)**, Italy

Person	Contribution
Ladislas Meduna	Convulsive therapy (camphor/ metrazol, 1934)
Manfred Sakel	Insulin coma therapy (1933)
Egas Moniz	**Prefrontal lobotomy** – Nobel 1949
John Cade	**Lithium** for mania (1949)
Delay & Deniker	**Chlorpromazine (1952)** – first antipsychotic
Maxwell Jones	**Therapeutic community**
Aaron Beck	**Cognitive therapy (CBT)**
B. F. Skinner / Pavlov	Operant / classical conditioning

- **India** – first asylums set up under British rule in the **18th century (Bombay, Calcutta, Madras)**; **Indian Lunacy Act 1912** → **Mental Health Act 1987** → **Mental Healthcare Act 2017**; **NMHP 1982**; **DMHP 1996**; **National Mental Health Policy 2014**; **Tele-MANAS 2022**.

1.3 Classification of Mental Disorders

System	By	Current
ICD (International Classification of Diseases)	**WHO**	**ICD-11 (effective Jan 2022)**; mental disorders – **Chapter 6** (ICD-10 – Chapter V, F00–F99)
DSM (Diagnostic & Statistical Manual)	**American Psychiatric Association**	**DSM-5 (2013), DSM-5-TR (2022)**

- **Old classification** – **psychosis** (loss of contact with reality, **no insight** – schizophrenia, mania) vs **neurosis** (contact with reality maintained, **insight present** – anxiety, OCD, phobia).
- **Organic** (identifiable brain cause – delirium, dementia) vs **functional** (no structural cause).

1.4 Psychiatric Signs & Symptoms (Very Important)

Disorders of perception

Term	Meaning
Illusion	**Misinterpretation of a real external stimulus** (rope seen as snake)
Hallucination	**Perception without external stimulus**; **auditory – most common (schizophrenia)**; **visual – organic (delirium, DT)**; **tactile – formication (cocaine "bugs")**; olfactory/ gustatory (temporal lobe epilepsy)
Hypnagogic / hypnopompic	Hallucinations while falling asleep / on waking (normal, narcolepsy)
Depersonalisation	Feeling **oneself** is unreal/ changed
Derealisation	Feeling **surroundings** are unreal
Déjà vu / Jamais vu	Familiar feeling for new / unfamiliar feeling for familiar

Disorders of thought

Term	Meaning
Delusion	**Fixed, false, unshakable belief**, not in keeping with patient's socio-cultural background; **persecutory – most common**; grandiosity (mania), nihilistic & guilt (severe depression), jealousy (**Othello syndrome – alcohol**), erotomanic (**de Clérambault's**), reference, control
Obsession	**Recurrent, intrusive, unwanted thought** recognised as **own** & irrational (OCD)
Flight of ideas	Rapid shift of ideas with **some connection** – **mania**
Loosening of association (derailment)	No logical connection – **schizophrenia**
Tangentiality	Never reaches the goal
Circumstantiality	Reaches goal after many unnecessary details
Thought block	Sudden stop in thought – **schizophrenia**
Neologism	**New words** made up
Word salad	Incoherent mixture of words
Clang association	Words chosen by **sound (rhyming)** – **mania**
Perseveration	Persistent repetition of same response – **organic**
Echolalia / echopraxia	Repeating words / actions of others – **catatonia**
Poverty of speech/ alogia	Negative symptom
Confabulation	**Filling memory gaps with imaginary events** – **Korsakoff**

Disorders of mood/ affect & behaviour

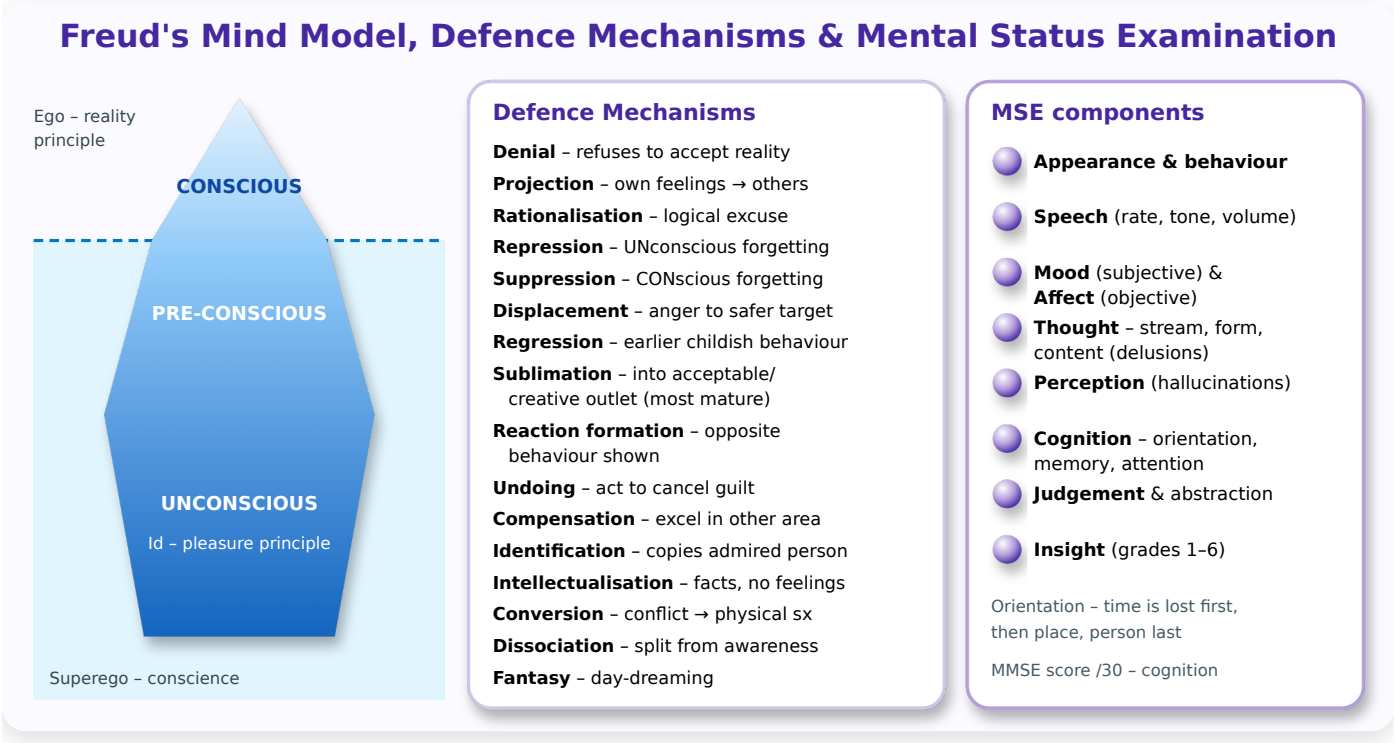
Term	Meaning
Mood	Sustained, pervasive emotion (**subjective** – "climate")
Affect	Observable expression of emotion (**objective** – "weather") – **blunted, flat (schizophrenia), labile, inappropriate (incongruent)**
Euphoria / elation / exaltation / ecstasy	Increasing degrees of happy mood (mania)
Anhedonia	**Loss of pleasure** – depression, schizophrenia
Ambivalence	Co-existence of opposing feelings (schizophrenia)
Alexithymia	Inability to describe emotions
Catatonia	**Stupor, mutism, negativism, posturing, waxy flexibility (catalepsy), echolalia/ echopraxia, automatic obedience**
Stereotypy / mannerism	Repetitive purposeless movement / goal-directed odd movement
Psychomotor retardation / agitation	Depression

Disorders of memory & consciousness

- **Amnesia** – **retrograde** (before event), **anterograde** (after event – new learning); **Korsakoff – anterograde + confabulation**.
- **Orientation** – **time lost first, then place, person last** (organic).
- **Insight** – awareness of illness (**grade 1 complete denial → grade 6 true emotional insight**); **psychosis – poor insight**.
- **Judgement** – ability to assess situation & act appropriately.

1.5 Personality & Theories

- **Freud's structure of personality** – **Id** (instincts, **pleasure principle**, unconscious), **Ego** (**reality principle**, mediator), **Superego** (**conscience**, morals).
- **Levels of consciousness** – conscious, preconscious, unconscious ("iceberg").
- **Psychosexual stages** – oral, anal, phallic, latency, genital; **fixation**.



Freud's model, common defence mechanisms & MSE components

- **Defence (ego defence) mechanisms** – unconscious strategies to reduce anxiety:

Mechanism	Example
Denial	Alcoholic says "I don't have a drinking problem" (**most primitive**)
Projection	Blaming others for own unacceptable feelings ("He hates me" when I hate him) – **paranoia**
Rationalisation	Logical-sounding excuse ("I failed because the paper was out of syllabus")
Repression	**Unconscious** forgetting of painful memory (**basic defence**)
Suppression	**Conscious** pushing away of thoughts (only conscious one)
Displacement	Angry at boss → shouts at child
Regression	Return to earlier developmental behaviour (hospitalised child bedwetting)

Mechanism	Example
Sublimation	Unacceptable drive → **socially acceptable** (aggression → boxing) – **most mature/ healthy**
Reaction formation	Shows **opposite** behaviour (dislikes someone but is excessively nice)
Undoing	Act to cancel a guilty act (gift after quarrel) – **OCD**
Compensation	Weak in studies → excels in sports
Identification / introjection	Adopting traits of admired person / incorporating values
Intellectualisation	Talking about facts to avoid feelings
Conversion	Psychological conflict → **physical symptom** (paralysis)
Dissociation	Separation of thoughts/ memory from awareness (amnesia, fugue)
Fantasy	Day-dreaming

- **Other theories** – **Erikson** (psychosocial), **Sullivan** (interpersonal), **Maslow** (hierarchy of needs), **behavioural** (Pavlov, Skinner), **cognitive** (Beck), **biological (neurotransmitters)**.
- **Neurotransmitters** – **dopamine ↑ – schizophrenia, mania**; **↓ dopamine – Parkinsonism**; **serotonin ↓ & norepinephrine ↓ – depression**; **GABA ↓ – anxiety**; **acetylcholine ↓ – Alzheimer's**; **glutamate**.

1.6 Mental Health Team & Nurse's Role

- **Team** – psychiatrist, **psychiatric nurse**, clinical psychologist, psychiatric social worker, occupational therapist, counsellor, recreational therapist, family.
- **Roles of psychiatric nurse** – **caregiver**, **therapeutic communicator**, **counsellor**, **health educator**, **milieu manager**, **advocate**, **case manager**, **socialising agent**, **surrogate parent**, **medication manager**, crisis interventionist, researcher, liaison/ consultant.
- **Principles of psychiatric nursing** – **accept patient as he/ she is**, **use self therapeutically**, **consistency**, **reassurance**, **avoid physical & verbal force**, **set limits**, **confidentiality**, **observe & record behaviour accurately**, **honesty**, **non-judgemental attitude**, maintain dignity, understand behaviour as meaningful.

PYQ style

- WHO mental health includes – **coping with normal stresses, working productively**.
- Removed chains from mentally ill – **Philippe Pinel**.
- Coined schizophrenia – **Bleuler**; first-rank symptoms – **Kurt Schneider**.
- Father of psychoanalysis – **Freud**; mother of psychiatric nursing – **Peplau**.
- First American psychiatric nurse – **Linda Richards**.
- ECT introduced by – **Cerletti & Bini (1938)**; lithium – **John Cade**; first antipsychotic – **chlorpromazine**.
- ICD by – **WHO (ICD-11)**; DSM by – **American Psychiatric Association**.
- Perception without stimulus – **hallucination**; misinterpretation of real stimulus – **illusion**.
- Most common hallucination in schizophrenia – **auditory**; in delirium – **visual**.
- Fixed false belief – **delusion**; most common – **persecutory**.
- Flight of ideas & clang association – **mania**; loosening of association & thought block – **schizophrenia**.
- Confabulation – **Korsakoff's psychosis**.
- Waxy flexibility – **catatonia**.
- Conscious defence mechanism – **suppression**; most mature – **sublimation**.
- Alcoholic denying problem – **denial**; blaming others – **projection**.
- Orientation lost first – **time**.
- Dopamine excess – **schizophrenia**; serotonin deficiency – **depression**; ACh deficiency – **Alzheimer's**.

2 Psychiatric Assessment, Therapeutic Communication & Nurse–Patient Relationship

2.1 Psychiatric Assessment

- **Psychiatric history** – identifying data, **chief complaints (in patient's own words)**, history of present illness, past psychiatric & medical history, **family history** (mental illness, suicide, substance use), **personal history** (birth, milestones, education, occupation, marital, sexual), **premorbid personality**, substance use, forensic history.
- **Informants** – patient + relatives (reliability & adequacy noted).

Mental Status Examination (MSE)

Component	What is assessed
General appearance & behaviour	Grooming, dress, posture, eye contact, rapport, psychomotor activity, abnormal movements
Speech	**Rate, rhythm, volume, tone, quantity** (pressure of speech – mania; poverty – depression/ schizophrenia)
Mood (subjective – "How do you feel?") & **affect** (objective – range, intensity, appropriateness)	Depressed, elated, anxious, irritable; flat/ blunted
Thought	**Stream/ form** (flight of ideas, loosening), **content** (**delusions, obsessions, suicidal/ homicidal ideas**, phobias)
Perception	Hallucinations, illusions, depersonalisation
Cognitive functions	**Consciousness**, **orientation (time, place, person)**, **attention & concentration (serial 7s/ 3s, digit span)**, **memory (immediate – digit span; recent – 24 hrs events; remote)**, intelligence, **abstract thinking (proverbs, similarities)**
Judgement	Personal, social, **test judgement** ("What will you do if you see a house on fire?")
Insight	Awareness of illness – **grades 1–6**

- **Rating scales** – **MMSE (Folstein) – cognition, max 30 (≤ 23/ 24 = impairment)**; **HAM-D (Hamilton depression)**, **BDI (Beck Depression Inventory)**, **PHQ-9 (depression screening)**, **GAD-7**, **YMRS (Young Mania)**, **BPRS (Brief Psychiatric Rating Scale)**, **PANSS (schizophrenia)**, **Y-BOCS (OCD)**, **CIWA-Ar (alcohol withdrawal)**, **CAGE & AUDIT (alcohol)**, **SAD PERSONS (suicide risk)**, **AIMS (tardive dyskinesia)**, **GCS**.
- **Investigations** – CBC, LFT, RFT, TSH, **serum lithium**, drug screen, EEG, CT/ MRI, psychological tests (IQ – **WAIS, Binet**; personality – **MMPI, Rorschach inkblot, TAT**).

2.2 Therapeutic Communication

D Therapeutic communication

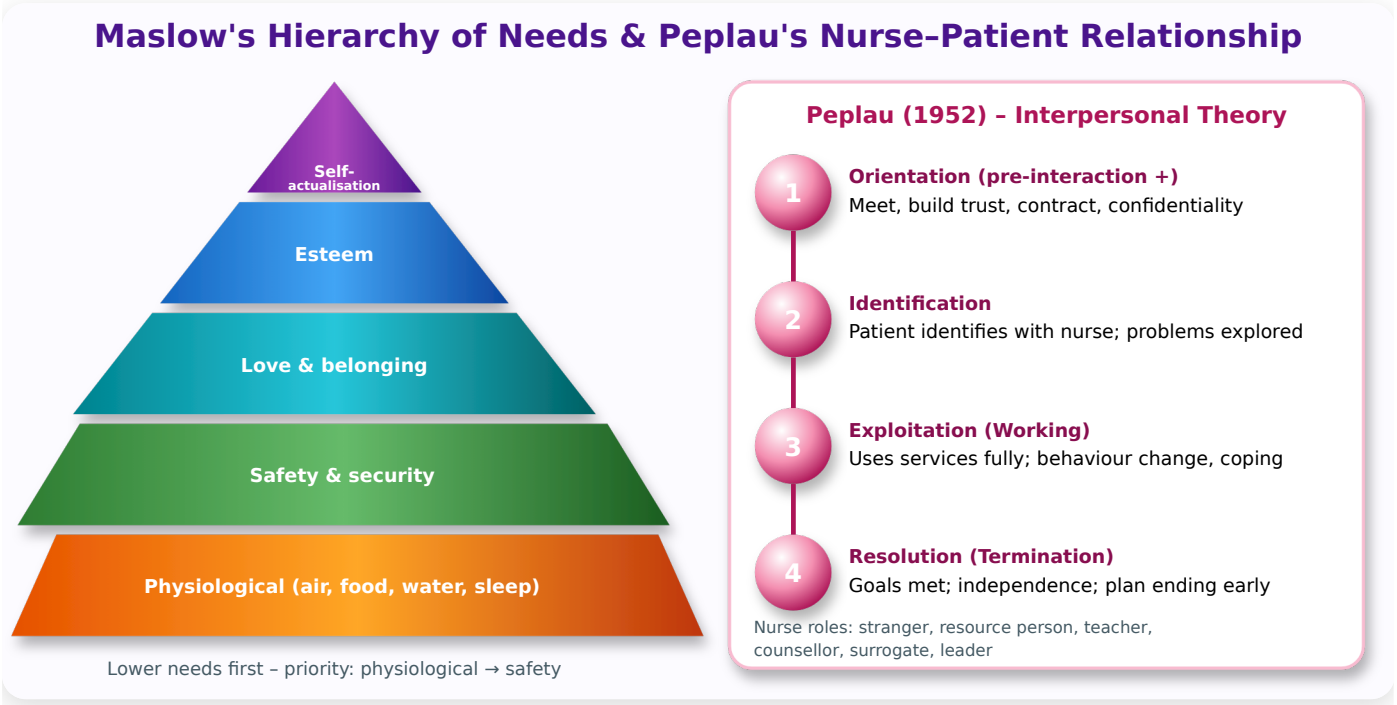
Purposeful, goal-directed, patient-centred interaction that helps the patient **express feelings, understand problems & find solutions**; the nurse uses **self as a therapeutic tool**.

Therapeutic techniques	Example
Active listening	Attentive, eye contact, nodding (**SOLER** – Sit squarely, Open posture, Lean forward, Eye contact, Relaxed)
Broad openings	"What would you like to talk about today?"
Open-ended questions	"How have you been feeling?"
Silence	Gives time to think/ express (therapeutic pause)
Restating	Repeating main idea – "You say you can't sleep."
Reflecting	Directing feelings back – "You seem angry."
Clarification	"I'm not sure I understand. Could you explain?"
Focusing	Concentrating on one point
Exploring	"Tell me more about that."
Validation/ consensual validation	Checking understanding
Giving recognition	"Good morning, Mr. Shah. You combed your hair."
Offering self	"I'll sit with you for a while."
Presenting reality	"I don't hear any voices; you are in the hospital."
Voicing doubt	"That seems hard to believe." (delusions – gently)
Summarising	Review of main points at end
Encouraging comparison, formulating a plan	–

Non-therapeutic (blocks)	Example
False reassurance	"Don't worry, everything will be fine."
Giving advice	"If I were you, I would..."
Approving/ disapproving, judging	"That's good/ bad."
Asking "Why?"	Makes patient defensive
Changing the subject	–
Belittling feelings	"Everybody feels that way."
Arguing / challenging	"You can't be the President."
Defending	"The doctor is very good."
Stereotyped/ cliché responses	"Keep your chin up."
Probing	Persistent questioning
Literal responses, interpreting	–

- **Johari window** (Joseph Luft & Harry Ingham) – **open (known to self & others), blind (others know, self doesn't), hidden (self knows, others don't), unknown**; self-awareness ↑ by **self-disclosure & feedback** → enlarges open area.
- **Self-awareness** of nurse – essential for therapeutic use of self.
- **Transference** – patient **unconsciously transfers feelings** about significant others onto the nurse; **counter-transference** – nurse's emotional reaction to patient (must be recognised – supervision).
- **Empathy (understanding patient's feelings from his perspective)** vs **sympathy** (sharing feelings – not therapeutic); **empathy – key to therapeutic relationship**.
- **Process recording** – verbatim record of nurse–patient interaction for analysis (student learning).

2.3 Nurse–Patient Relationship (Peplau)



Maslow's needs pyramid & Peplau's phases of nurse–patient relationship

Phase	Tasks
Pre-interaction (added by Travelbee/ others)	Nurse's **self-exploration** of feelings, fears; gather data; plan first meeting
Orientation (introductory)	**Introductions, establish trust & rapport**, **contract** (time, place, duration, purpose), **confidentiality**, **termination discussed from the start**, identify problems; patient may **test** the nurse
Working phase (identification + exploitation)	Explore stressors, **promote insight**, develop **coping**, problem solving, overcome resistance; **transference** may appear
Termination (resolution)	Review progress, **deal with feelings of separation/ loss** (sadness, anger, regression), evaluate goals, **plan for future/ discharge**; **termination should be planned & gradual**

- **Peplau's nurse roles** – **stranger, resource person, teacher, counsellor, surrogate, leader** (+ technical expert).

- **Characteristics of therapeutic relationship** – **trust, empathy, respect, genuineness, acceptance, non-judgemental, warmth, boundaries, confidentiality**.
- **Boundaries** – avoid personal/ social relationship, gifts, over-involvement; **professional relationship**.
- **Maslow – priority**: physiological → safety → love/ belonging → esteem → self-actualisation; in psychiatry, **safety (suicide, violence)** is a top priority after physiological needs.

2.4 Nursing Process in Psychiatry (examples)

Problem	Nursing diagnosis (NANDA)
Suicidal ideation	**Risk for suicide / self-directed violence**
Aggression	**Risk for other-directed violence**
Hallucinations	**Disturbed sensory perception**
Delusions	**Disturbed thought processes**
Withdrawal	**Social isolation**
Poor self-care	**Self-care deficit**
Low self-esteem	**Chronic low self-esteem**
Anxiety	**Anxiety (mild/ moderate/ severe/ panic)**
Non-compliance	**Ineffective health management**

- **Levels of anxiety (Peplau)** – **mild** (↑ alertness, learning enhanced), **moderate** (narrowed perception, selective attention), **severe** (very narrow, can't problem-solve), **panic** (disorganised, loss of control – **stay with patient, calm, simple directions, safe environment**).

PYQ style

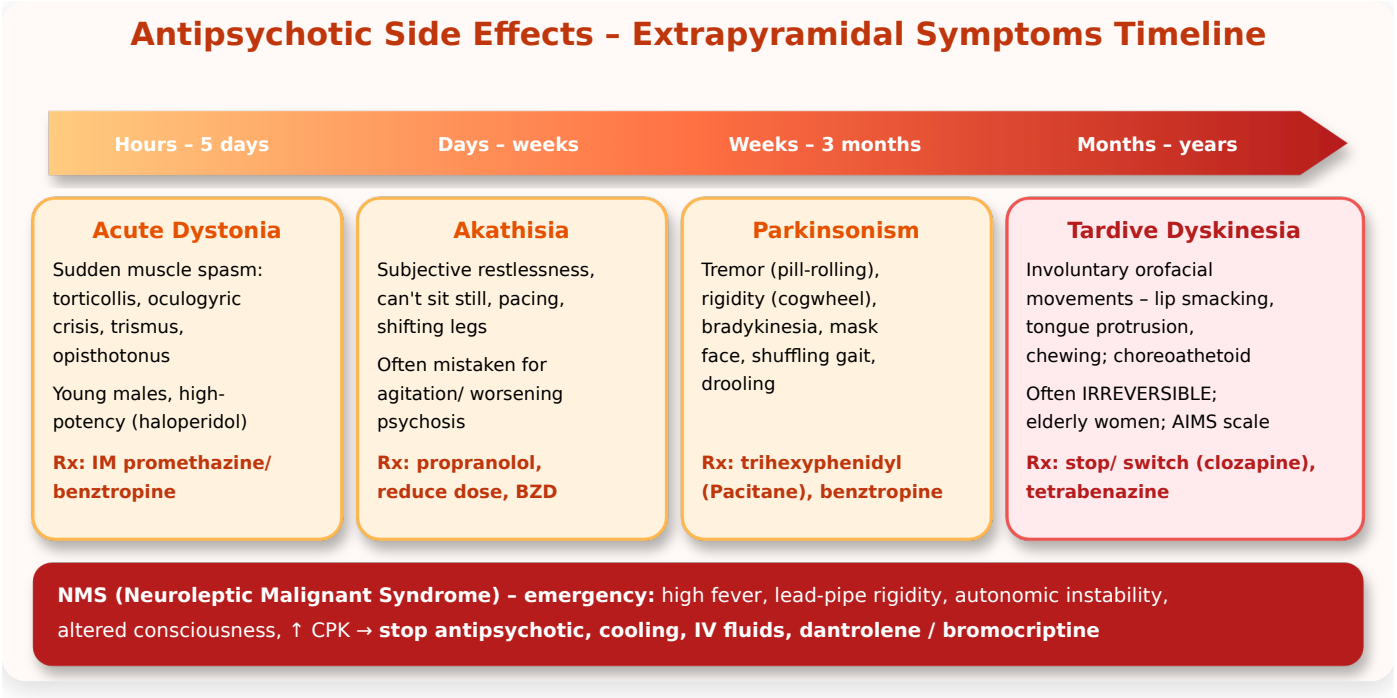
- MSE – mood is **subjective**, affect is **objective**.
- Test for attention/ concentration – **serial 7s / digit span**.
- Proverb interpretation tests – **abstract thinking**.
- "What will you do if you find a stamped letter?" – **test judgement**.
- MMSE maximum score – **30**.
- Suicide risk scale – **SAD PERSONS**; alcohol screening – **CAGE**; withdrawal – **CIWA-Ar**; TD – **AIMS**.
- Rorschach inkblot – **personality (projective) test**.
- Most therapeutic response to patient's feelings – **reflection/ open-ended**.
- "Don't worry, you'll be fine" – **false reassurance (non-therapeutic)**.
- Asking "why" is – **non-therapeutic**.
- Hallucinating patient – **present reality ("I don't hear voices")**.
- Johari window – **self-awareness**; quadrants – **open, blind, hidden, unknown**.
- Patient treats nurse like his mother – **transference**.
- Phases (Peplau) – **orientation → identification → exploitation → resolution**.
- Contract & trust established in – **orientation phase**.
- Termination discussed – **from the beginning (orientation)**.
- Panic anxiety – **stay with patient, calm simple directions**.

3 Psychopharmacology

3.1 Antipsychotics (Neuroleptics)

- **Mechanism** – **dopamine D2 receptor blockade** (mesolimbic → ↓ positive symptoms); atypicals also block **5-HT2A**.
- **Uses** – schizophrenia, mania, psychotic depression, delirium, agitation, Tourette's, hiccups (chlorpromazine), vomiting.

Class	Drugs	Key features
Typical (1st generation) – low potency	**Chlorpromazine (Largactil)** – first (1952), **thioridazine**	Sedation, **anticholinergic**, **hypotension**, photosensitivity, **less EPS**; thioridazine – retinitis pigmentosa, QT
Typical – high potency	**Haloperidol (Serenace)**, **fluphenazine**, trifluoperazine	**More EPS**, less sedation; haloperidol – IM for acute agitation
Atypical (2nd generation)	**Risperidone**, **olanzapine**, **quetiapine**, **aripiprazole**, ziprasidone, **clozapine**, amisulpride	Fewer EPS, effective on **negative symptoms**; **metabolic syndrome (weight gain, diabetes, dyslipidaemia – olanzapine, clozapine)**; **risperidone – hyperprolactinaemia (galactorrhoea, amenorrhoea)**; **aripiprazole – partial agonist (least metabolic)**
Clozapine	–	**Treatment-resistant schizophrenia**, ↓ suicide; **agranulocytosis (1 %) – regular WBC/ ANC monitoring (weekly first 18 weeks)**; **seizures**, **sialorrhoea (hypersalivation)**, myocarditis, weight gain, constipation
Depot (long-acting injectable)	**Fluphenazine decanoate**, **haloperidol decanoate**, flupentixol, **paliperidone palmitate**, risperidone LAI	**Non-compliant patients** – deep IM (Z-track) every **2–4 weeks**



Extrapyramidal side effects timeline & NMS

Side effects

- **Extrapyramidal symptoms (EPS)** – **acute dystonia** (hours–days; **IM promethazine/ diphenhydramine/ benztropine**), **akathisia** (days–weeks; **propranolol**), **parkinsonism** (weeks; **trihexyphenidyl/ benztropine**), **tardive dyskinesia** (months–years; **irreversible; lip smacking, tongue movements; AIMS; stop/ switch to clozapine, tetrabenazine/ valbenazine**; anticholinergics worsen TD).
- **Neuroleptic Malignant Syndrome (NMS)** – **rare, life-threatening**; **hyperthermia, lead-pipe rigidity, autonomic instability (labile BP, tachycardia, sweating), altered consciousness, ↑ CPK, leukocytosis, myoglobinuria**; Rx – **stop antipsychotic immediately**, **cooling, IV fluids, dantrolene (muscle relaxant), bromocriptine (dopamine agonist)**, ICU.
- **Anticholinergic** – dry mouth, blurred vision, constipation, **urinary retention**, tachycardia (**"can't see, can't pee, can't spit, can't..."**).
- **Anti-adrenergic** – **orthostatic hypotension** (rise slowly).
- **Endocrine** – hyperprolactinaemia, weight gain; **metabolic** – diabetes, lipids.
- **Others** – sedation, **photosensitivity (chlorpromazine – use sunscreen)**, **lowered seizure threshold**, **QT prolongation**, agranulocytosis, cholestatic jaundice (chlorpromazine), sexual dysfunction.

3.2 Antidepressants

Class	Drugs	Key points
SSRIs (Selective Serotonin Reuptake Inhibitors) – **first-line**	**Fluoxetine** (first, **longest half-life**), **sertraline** (safe in cardiac/ breastfeeding), **escitalopram**, paroxetine, fluvoxamine (OCD)	GI upset, **sexual dysfunction**, insomnia/ agitation, headache; **safe in overdose**; **SIADH (elderly)**; discontinuation syndrome (paroxetine)
SNRIs	**Venlafaxine, duloxetine**	Also for pain (duloxetine – diabetic neuropathy); ↑ BP (venlafaxine)

Class	Drugs	Key points
TCAs (Tricyclic)	**Imipramine** (first – 1957), **amitriptyline**, clomipramine (OCD), nortriptyline	**Anticholinergic**, sedation, **orthostatic hypotension**, weight gain, **cardiotoxic in overdose (QRS widening, arrhythmia) – most dangerous in overdose**; amitriptyline – neuropathic pain, migraine; imipramine – enuresis
MAOIs	**Phenelzine, tranylcypromine**, moclobemide (reversible)	**Hypertensive crisis ("cheese reaction") with tyramine-rich foods – aged cheese, wine/ beer, pickled/ smoked fish, soy sauce, fava beans, chocolate**; **serotonin syndrome with SSRIs – 2-week washout** (5 weeks after fluoxetine)
Atypical	**Mirtazapine** (↑ appetite, sedation – at night), **bupropion** (smoking cessation, **no sexual dysfunction**, ↓ seizure threshold), trazodone (priapism), agomelatine	–

- **Onset of action – 2–4 weeks (full effect 4–6 weeks)**; continue **6–12 months after remission** (first episode).
- **Suicide risk ↑ in early treatment** – energy returns before mood lifts ("**paradoxical**") – **close observation**, dispense limited quantity.
- **Serotonin syndrome** – **agitation, confusion, hyperthermia, sweating, tachycardia, myoclonus, hyperreflexia, tremor, diarrhoea** (vs NMS – rigidity, slower onset); Rx – stop drug, **cyproheptadine**, supportive.
- **Switching to mania** in bipolar patients.

3.3 Mood Stabilisers

Lithium carbonate

- **Drug of choice for bipolar disorder (prophylaxis & acute mania)**; **reduces suicide**.
- **Therapeutic range – 0.6–1.2 mEq/L** (maintenance **0.6–1.0**; acute 0.8–1.2); **toxic > 1.5 mEq/L**; **severe > 2.5 (dialysis)**.
- **Serum level 12 hrs after last dose** (trough); weekly initially, then 3-monthly.
- **Narrow therapeutic index**; onset **7–14 days** (antipsychotic/ BZD added in acute mania).
- **Side effects** – **fine tremor (hands), polyuria & polydipsia (nephrogenic DI)**, nausea, metallic taste, **weight gain**, **hypothyroidism (goitre)**, acne, **leucocytosis**, **Ebstein's anomaly (1st trimester – teratogenic)**, renal impairment, T-wave changes.
- **Toxicity** – **coarse tremor, vomiting, diarrhoea, ataxia, slurred speech, drowsiness, confusion → seizures, coma, arrhythmia**; Rx – **stop lithium**, IV NS, **haemodialysis (most effective)**.
- **Nursing/ teaching** – **adequate fluids (2–3 L/day) & normal salt intake** (↓ sodium → ↑ lithium reabsorption → toxicity); **avoid dehydration (vomiting, diarrhoea, sweating, fever)**; **avoid diuretics (thiazides), NSAIDs, ACE inhibitors** (↑ lithium level); take with meals; regular serum levels, **thyroid & renal function tests**; recognise toxicity; contraception.

Anticonvulsant mood stabilisers

Drug	Key points
Sodium valproate (divalproex)	Rapid cycling, mixed mania; **hepatotoxicity, pancreatitis, weight gain, hair loss, tremor, thrombocytopenia, NTD (teratogenic), PCOS**; LFT monitoring
Carbamazepine	**Agranulocytosis/ aplastic anaemia**, **SJS (HLA-B*1502)**, hyponatraemia, enzyme inducer (↓ OCP effect)
Lamotrigine	Bipolar depression; **SJS rash (slow titration)**

3.4 Anxiolytics & Hypnotics

Class	Drugs	Key points
Benzodiazepines (BZD)	**Diazepam, lorazepam** (preferred in liver disease – no active metabolite; IM/ IV), **alprazolam**, clonazepam, **chlordiazepoxide** (alcohol withdrawal), midazolam, nitrazepam	**Enhance GABA**; anxiety, insomnia, **alcohol withdrawal**, seizures, **catatonia (lorazepam)**, premedication; **sedation, ataxia, dependence & withdrawal (seizures), tolerance, paradoxical agitation (elderly), respiratory depression with alcohol**; **antidote – flumazenil**; short-term use (2–4 weeks)
Buspirone	–	**Non-sedating, non-addictive** anxiolytic; onset 2–3 weeks
Z-drugs	**Zolpidem**, zopiclone	Insomnia
Others	Propranolol (performance anxiety – tremor, palpitations), hydroxyzine, melatonin	–

3.5 Other Psychotropics

Group	Drugs
Anticholinergics (antiparkinsonian)	**Trihexyphenidyl (benzhexol/ Pacitane)**, **benztropine**, **promethazine** – for EPS (**abuse potential – euphoria**)
Anti-dementia	**Cholinesterase inhibitors – donepezil, rivastigmine, galantamine**; **NMDA antagonist – memantine**
ADHD	**Methylphenidate** (stimulant – anorexia, insomnia, growth delay; give in morning), **atomoxetine**
Alcohol dependence	**Disulfiram** (aldehyde dehydrogenase inhibitor – **disulfiram-ethanol reaction**: flushing, vomiting, palpitation, hypotension – **never give without patient's knowledge**; avoid alcohol in cough syrups, aftershave), **naltrexone** (↓ craving – opioid antagonist), **acamprosate** (↓ craving), baclofen, topiramate
Opioid dependence	**Methadone, buprenorphine** (OST), **naltrexone**, clonidine (withdrawal); overdose – **naloxone**
Nicotine dependence	**NRT (gum, patch)**, **varenicline**, bupropion

3.6 Nurse's Responsibilities in Psychopharmacology

- **Ensure medication is swallowed** (check mouth – **"cheeking"/ hoarding** for suicide); **liquid forms** if suspected.
- **Observe for therapeutic & side effects**; vital signs (**BP lying & standing**), **weight, blood sugar, lipids**, **WBC (clozapine)**, **lithium levels**.
- **Patient & family education** – purpose, **delay in onset**, **don't stop abruptly**, side effects, **drug interactions**, **avoid alcohol**, **driving caution**, diet restrictions (MAOI), fluid/ salt (lithium), **sun protection (chlorpromazine)**.
- **Manage side effects** – dry mouth (sips of water, sugar-free gum), **constipation (fibre, fluids)**, **orthostatic hypotension (rise slowly, dangle legs)**, weight gain (diet, exercise).
- **Compliance** – depot injections, simplify regimen, involve family, psychoeducation.
- **Elderly** – start low, go slow.

PYQ style

- First antipsychotic – **chlorpromazine**; mechanism – **D2 blockade**.
- Agranulocytosis – **clozapine (WBC monitoring)**; treatment-resistant schizophrenia – **clozapine**.
- Hyperprolactinaemia – **risperidone**; weight gain/ diabetes – **olanzapine**.
- Depot antipsychotic – **fluphenazine decanoate**.
- Acute dystonia Rx – **IM promethazine/ benztropine**; akathisia – **propranolol**; parkinsonism – **trihexyphenidyl**.
- Irreversible EPS – **tardive dyskinesia**; scale – **AIMS**.
- NMS – **hyperthermia, rigidity, ↑ CPK**; Rx – **dantrolene, bromocriptine**.
- First-line antidepressant – **SSRI**; longest half-life – **fluoxetine**.
- Most dangerous in overdose – **TCAs (cardiotoxic)**.
- Cheese reaction – **MAOI + tyramine → hypertensive crisis**.
- Antidepressant onset – **2–4 weeks**; suicide risk – **early recovery phase**.
- Serotonin syndrome antidote – **cyproheptadine**.
- Lithium therapeutic level – **0.6–1.2 mEq/L**; toxic – **> 1.5**.
- Lithium toxicity first signs – **coarse tremor, vomiting, diarrhoea**.
- Lithium teaching – **adequate fluids & normal salt**; teratogen – **Ebstein's anomaly**.
- Lithium side effect – **hypothyroidism, polyuria, fine tremor**.
- BZD antidote – **flumazenil**; preferred BZD in liver disease – **lorazepam**.
- Non-addictive anxiolytic – **buspirone**.
- Disulfiram reaction – **with alcohol (flushing, vomiting)**.
- Alzheimer's drug – **donepezil**; ADHD – **methylphenidate**.

4 ECT, Psychotherapies & Therapeutic Modalities

4.1 Electroconvulsive Therapy (ECT)

D ECT

Induction of a generalised (grand mal) seizure by passing a controlled **electric current through the brain** under **general anaesthesia & muscle relaxation (modified ECT)**. Introduced by **Cerletti & Bini (1938)**. Exact mechanism unclear – ↑ neurotransmitter sensitivity (monoamines), neurotrophic effects.

- **Indications** – **severe depression with suicidal risk/ refusal to eat (most common & most effective indication)**, **psychotic depression**, **catatonia (stupor)**, **treatment-resistant depression**, **acute mania** (refractory), **schizophrenia with catatonia/ severe excitement**, **NMS**, depression in **pregnancy** (safer than drugs), postpartum psychosis.
- **Contraindications** – **no absolute contraindication** now; **relative – raised intracranial pressure (brain tumour – most important)**, recent MI (< 3 months), recent stroke, aneurysm, severe HTN, retinal detachment, pheochromocytoma, high anaesthetic risk.
- **Course** – **2–3 times/week**, total **6–12 sessions**.
- **Electrode placement** – **bilateral (bitemporal)** – more effective, more memory loss; **unilateral (non-dominant)** – less cognitive side effects.
- **Adequate seizure** – motor seizure **≥ 20–25 seconds** (EEG seizure longer); monitored by **cuff method** (BP cuff inflated on one limb – observe convulsion in that limb).

Pre-ECT nursing care

- **Informed written consent** (patient/ nominated representative – **MHCA 2017**).
- **NPO (nil by mouth) for 6–8 hours** (prevents aspiration).
- **Pre-anaesthetic check-up**, investigations (CBC, ECG, X-ray, fundus if needed).
- **Remove dentures, jewellery, hairpins, contact lenses, nail polish/ make-up** (to observe cyanosis); **hair clean & dry, no oil** (better electrode contact).
- **Empty bladder & bowel** (incontinence during seizure).
- **Vital signs** recorded; loose clothing; ID band.
- **Premedication** – **atropine/ glycopyrrolate** (↓ secretions, prevent vagal bradycardia).
- Reassure patient (anxiety).

During ECT

- **Supine** position, **IV line**; **anaesthesia – thiopentone/ propofol/ methohexital**; **muscle relaxant – succinylcholine (suxamethonium)** → prevents fractures; **100 % O_2 with bag & mask**; **mouth gag/ bite block** (protects teeth & tongue); electrodes with conducting jelly; **ECT shock**; observe & time seizure; ventilate till spontaneous breathing.

Post-ECT care

- **Recovery position (lateral)**, **airway**, suction, O_2, **vital signs every 15 min** till stable, side rails up.
- **Re-orient** patient (confusion common), stay with patient.
- **Oral fluids/ food** only after full consciousness & **gag reflex returns**.
- Observe for **headache, confusion, memory loss, muscle pain (succinylcholine), nausea**.

Side effects

- **Memory impairment – most common** (**retrograde & anterograde amnesia**, usually **temporary – recovers in weeks**), **post-ictal confusion**, **headache**, **muscle aches**, nausea; rare – fractures (unmodified), prolonged seizure, cardiac arrhythmia, status epilepticus, death (~ anaesthesia risk).
- **MHCA 2017 (Sec 95)** – **ECT only with anaesthesia & muscle relaxant ("modified")**; **ECT prohibited in minors** (except with permission of Mental Health Review Board under specific conditions).

4.2 Other Somatic Therapies

Therapy	Key points
rTMS (repetitive transcranial magnetic stimulation)	Depression – non-invasive, no anaesthesia
Insulin coma therapy (Sakel)	Historical (obsolete)
Psychosurgery (lobotomy – Moniz)	Rare now – refractory OCD (cingulotomy)
Light therapy (phototherapy)	**Seasonal affective disorder (SAD)**
Sleep deprivation	Depression (short-term)
Deep brain stimulation, VNS	Refractory depression/ OCD

4.3 Psychotherapies

Therapy	Founder	Key points/ use
Psychoanalysis	**Sigmund Freud**	**Free association, dream analysis ("royal road to the unconscious")**, interpretation, transference; long-term; neurosis
Cognitive Behaviour Therapy (CBT)	**Aaron Beck** (cognitive), Albert Ellis (**REBT**)	Identifies & modifies **negative automatic thoughts/ cognitive distortions**; **depression, anxiety, OCD, phobias**, eating disorders; **cognitive triad of depression (negative view of self, world, future)**
Behaviour therapy	Pavlov, Skinner, **Wolpe**	Based on learning theory
– **Systematic desensitisation**	**Joseph Wolpe**	**Phobias** – relaxation + gradual exposure (hierarchy)
– **Flooding / implosion**	–	Sudden intense exposure (phobia)
– **Exposure & response prevention (ERP)**	–	**OCD** – treatment of choice (with SSRIs)
– **Aversion therapy**	–	**Alcoholism**, sexual deviations (unpleasant stimulus paired)

Therapy	Founder	Key points/ use
– **Token economy**	–	**Operant conditioning – rewards** – chronic schizophrenia, intellectual disability, children
– **Biofeedback**, **relaxation (Jacobson's progressive muscle relaxation)**, **assertiveness training**, **modelling**, **social skills training**	–	Anxiety, stress
Group therapy	**Pratt (1905)**, Yalom	6–10 members; **Yalom's therapeutic factors – universality, instillation of hope, altruism, imparting information, catharsis, group cohesiveness, interpersonal learning**
Psychodrama	**J. L. Moreno**	Acting out conflicts
Family therapy	Bowen, Minuchin, Satir	Family as system; **expressed emotion (EE) – high EE ↑ relapse in schizophrenia**
Marital/ couple therapy	–	–
Interpersonal therapy (IPT)	Klerman	Depression
Supportive psychotherapy	–	Reassurance, guidance, ventilation
Hypnosis	(Mesmer)	Suggestion
Play therapy	–	**Children** – express feelings through play
Crisis intervention	**Gerald Caplan**	**Short-term (crisis resolves in 4–6 weeks)**; goal – **return to pre-crisis level of functioning**; types – **maturational (developmental), situational, adventitious (disaster)**
Counselling	**Carl Rogers – client-centred therapy**	Unconditional positive regard, empathy, genuineness
Logotherapy	Viktor Frankl	Meaning in life
Gestalt therapy	Fritz Perls	Here-and-now

4.4 Milieu Therapy & Rehabilitative Activities

- **Milieu therapy (therapeutic community)** – **Maxwell Jones**; **use of the total environment (people, setting, activities) as a therapeutic tool**; **patient participation in ward management (community meetings)**, democratic, open communication, **24-hr living-learning**; **nurse – milieu manager**.
- **Characteristics of therapeutic milieu** – safety, structure, norms, **limit setting**, open communication, balance, **involvement**, validation, acceptance.
- **Occupational therapy** – purposeful activities to improve function (crafts, gardening); **vocational rehabilitation**.
- **Recreational therapy** – games, music, dance; **activity therapy**.
- **Bibliotherapy** (reading), **art therapy**, **music therapy**, **yoga & meditation**.
- **Psychoeducation** – patient & family about illness, medication, relapse signs (**↓ relapse**).
- **Social skills training**, **day care centres**, **half-way homes**, **sheltered workshops**, **long-stay homes**.

4.5 Restraint & Seclusion

- **Last resort** – only when **imminent danger to self/ others** and less restrictive measures (**verbal de-escalation, PRN medication**) failed.
- **MHCA 2017 (Sec 97)** – **physical restraint only when necessary to prevent imminent harm**, by order of psychiatrist, **not as punishment**, minimum time, **monitored regularly**, informed to nominated representative, recorded; **seclusion & solitary confinement prohibited**; **chaining prohibited**.
- **Nursing** – adequate staff, explain reason, **check circulation & skin every 15 min**, vital signs, **offer fluids, toilet**, release one limb at a time/ ROM every 2 hrs, document, debrief.

PYQ style

- ECT introduced by – **Cerletti & Bini (1938)**.
- Most common indication of ECT – **severe depression with suicidal risk**.
- Most important (relative) contraindication – **raised ICP (brain tumour)**.
- NPO before ECT – **6–8 hours**.
- Premedication to reduce secretions – **atropine/ glycopyrrolate**.
- Muscle relaxant in ECT – **succinylcholine**; anaesthetic – **thiopentone/ propofol**.
- Most common side effect of ECT – **memory loss (temporary)**.
- ECT frequency – **2–3/week; 6–12 sessions**.
- Light therapy – **seasonal affective disorder**.
- CBT founder – **Aaron Beck**; cognitive triad – **negative view of self, world, future**.
- Systematic desensitisation – **Wolpe – phobias**.
- OCD treatment of choice – **ERP + SSRIs**.
- Aversion therapy – **alcoholism**; token economy – **operant conditioning**.
- Free association – **psychoanalysis (Freud)**.
- Crisis intervention – **Caplan; 4–6 weeks**.
- Therapeutic community – **Maxwell Jones**.
- Psychodrama – **Moreno**; client-centred therapy – **Carl Rogers**.
- High expressed emotion – **↑ relapse in schizophrenia**.
- Restraint – **last resort, check every 15 min**.

5 Schizophrenia & Other Psychotic Disorders

5.1 Schizophrenia – Basics

D Schizophrenia

A **severe, chronic psychotic disorder** characterised by **disturbances in thinking, perception, emotion, behaviour & volition**, with **loss of contact with reality** and **impaired functioning**, in **clear consciousness**. Term by **Eugen Bleuler (1908/1911)** – "split mind" (splitting of psychic functions, **NOT split personality**). Kraepelin – "**dementia praecox**".

- **Prevalence ~1 %** (lifetime); **onset – late adolescence/ early adulthood**: **males 15–25 yrs, females 25–35 yrs** (males earlier & worse prognosis); equal sex incidence.
- **Aetiology** – **genetic** (one parent – ~12 %, **both parents ~40–46 %**, **monozygotic twin ~48 %**), **dopamine hypothesis (excess dopamine in mesolimbic pathway – positive symptoms; ↓ in mesocortical – negative symptoms)**, glutamate (NMDA hypofunction), serotonin; **neurodevelopmental** (obstetric complications, winter births, prenatal infection); **ventricular enlargement**, ↓ frontal activity; **cannabis use**, urban living, migration, stress (**stress–diathesis model**); family – **high expressed emotion → relapse**; "**schizophrenogenic mother**" & **double-bind theory (Bateson)** – historical.

5.2 Clinical Features

- **Bleuler's 4 A's (fundamental symptoms)** – **A**ssociation disturbance (loosening), **A**ffective disturbance (**flat/ inappropriate affect**), **A**mbivalence, **A**utism (withdrawal into fantasy) (+ **A**ttention, **A**volition sometimes added).
- **Kurt Schneider's First-Rank Symptoms (FRS)** – highly suggestive (not pathognomonic):
  1. **Audible thoughts (thought echo – "Gedankenlautwerden")**
  2. **Voices arguing or discussing** (third person)
  3. **Voices commenting** on one's actions (running commentary)
  4. **Thought withdrawal**
  5. **Thought insertion**
  6. **Thought broadcasting** (thought diffusion)
  7. **Made feelings, made impulses, made volitional acts** (**passivity phenomena** – controlled by external force)
  8. **Somatic passivity**
  9. **Delusional perception**

Positive symptoms (excess/ distortion)	Negative symptoms ("5 A's")
Hallucinations (auditory – most common)	**Alogia** (poverty of speech)
Delusions (persecutory, reference, control)	**Avolition** (lack of motivation, apathy)
Disorganised speech (loosening, word salad, neologisms)	**Affective flattening/ blunting**
Disorganised/ bizarre behaviour, catatonic behaviour	**Anhedonia**

Positive symptoms (excess/ distortion)	Negative symptoms ("5 A's")
–	**Asociality** (social withdrawal), attention deficit
Respond better to antipsychotics; **better prognosis**	Respond poorly; **poor prognosis**, more disability

- **Diagnosis** – **ICD-10/ 11: symptoms for ≥ 1 month**; **DSM-5: ≥ 6 months total (≥ 1 month active phase)**; **< 1 month – acute & transient psychotic disorder (ICD)/ brief psychotic disorder (DSM)**; **1–6 months – schizophreniform (DSM)**.
- **Phases** – **prodromal** (social withdrawal, decline in functioning, odd beliefs) → **active (acute psychotic)** → **residual**.

5.3 Types (ICD-10 – still asked; ICD-11/ DSM-5 removed subtypes)

Type	Features
Paranoid	**Most common**; **prominent delusions (persecution/ grandeur) & auditory hallucinations**; personality relatively preserved; **later onset, better prognosis**; may be hostile/ suspicious
Hebephrenic (disorganised)	**Early onset (15–25 yrs)**; **disorganised speech & behaviour, inappropriate/ silly affect (giggling), mannerisms, grimacing**; **worst prognosis**
Catatonic	**Psychomotor disturbances – stupor/ excitement, mutism, negativism, posturing, waxy flexibility, echolalia, echopraxia, automatic obedience**; **best prognosis** (responds to **lorazepam & ECT**)
Undifferentiated	Doesn't fit any type
Residual	Negative symptoms after active phase
Simple	**Insidious negative symptoms without positive symptoms** – drifting, vagrancy
Post-schizophrenic depression	Depression after an episode

Prognosis

Good prognosis	Poor prognosis
Acute onset, late onset, precipitating stressor	**Insidious onset**, early onset
Female, married	Male, single
Positive symptoms, catatonic/ paranoid	**Negative symptoms**, hebephrenic/ simple
Mood symptoms present	Family history of schizophrenia
Good premorbid personality & social support	Poor premorbid adjustment, **high EE family**
Good compliance	Substance use, non-compliance, structural brain changes

- **Course** – ~1/3 recover substantially, ~1/3 intermittent relapses, ~1/3 chronic deterioration; **suicide risk ~5–10 %** (esp. young males after first episode, post-psychotic depression).

5.4 Treatment

- **Antipsychotics** (**typical & atypical**; **clozapine for treatment-resistant** – failed ≥ 2 antipsychotics); **depot** for non-compliance; **maintenance – at least 1–2 yrs after first episode, longer/ lifelong for**

relapses.

- **ECT** – catatonia, acute excitement, suicidal, resistant.
- **Psychosocial** – **psychoeducation (patient & family)**, **family therapy (↓ expressed emotion)**, **social skills training**, **vocational rehabilitation**, **token economy**, **CBT for psychosis**, **assertive community treatment**, day care, half-way homes, supported employment.

5.5 Nursing Care

Hallucinations

- **Observe for cues** (talking/ laughing to self, listening posture, sudden anger) – **ask directly**: "Are you hearing voices? What are they saying?" (check **command hallucinations** – risk of harm).
- **Don't argue or deny** patient's experience; **don't agree**; **present reality**: "I understand the voices seem real to you, but I don't hear them."
- **Focus on reality-based activities**, **distraction** (music, conversation, activities), **reduce environmental stimuli**, consistent staff.
- Teach **coping** – humming, talking to trusted person, earphones/ music, **voice dismissal**.
- **Safety** – command hallucinations ordering harm → close observation.

Delusions

- **Don't argue, challenge or try to convince** patient his delusion is false; **don't agree/ reinforce** either.
- **Focus on feelings behind delusion** ("You must feel frightened") & **reality**; **voice doubt** gently; **avoid whispering/ laughing** in front of patient (paranoid).
- **Build trust** – be **honest, consistent**, keep promises, **avoid touching suspicious patient without explanation**.
- **Persecutory delusion & food refusal ("poisoned food")** – **sealed/ packaged food, let patient choose own tray, family-brought food**, eat in common dining room.
- **Engage in reality-based activities**; don't focus on delusional content.

Withdrawal/ social isolation & negative symptoms

- **Brief, frequent contacts**, accept silence, gradually involve in **one-to-one → small group activities**, **simple tasks**, **positive reinforcement**, assist **self-care (ADL schedule)**.

Catatonia

- **Physical needs** – nutrition (tube feeding if needed), fluids, **elimination**, **skin care, position change 2-hrly (pressure sores)**, **ROM exercises**, mouth & eye care; **talk to patient (they may be aware)**; **lorazepam, ECT**; watch for **sudden excitement (catatonic excitement)**.

General

- **Medication compliance** (check swallowing), side effects (EPS, NMS, metabolic), **suicide & violence risk**, **family psychoeducation**, rehabilitation, **relapse prevention (early warning signs – sleep disturbance, withdrawal, suspiciousness)**.

5.6 Other Psychotic Disorders

Disorder	Key points
Persistent delusional disorder	**Non-bizarre fixed delusion ≥ 3 months (ICD) / ≥ 1 month (DSM)**; **otherwise functioning normally**; types – **persecutory (most common)**, **jealous (Othello syndrome – conjugal paranoia, alcohol)**, **erotomanic (de Clérambault – believes someone of higher status loves them)**, grandiose, **somatic (delusional parasitosis)**; Rx – antipsychotics
Acute & transient psychotic disorder (ATPD) / brief psychotic disorder	Acute onset (≤ 2 weeks), often after stressor, **< 1 month (DSM)/ < 3 months (ICD)**, full recovery; common in India
Schizoaffective disorder	Schizophrenia symptoms **+ prominent mood (manic/ depressive) episode** concurrently
Induced delusional disorder (Folie à deux)	**Shared psychotic disorder** – delusion transferred from dominant (primary) to dependent person; **separate them**
Postpartum (puerperal) psychosis	**Within 2–4 weeks of delivery**; confusion, hallucinations, **risk of infanticide & suicide** – **psychiatric emergency**; mostly bipolar; Rx – antipsychotics, lithium, **ECT**; supervised contact with baby
Capgras syndrome	Delusion that a familiar person is replaced by an **impostor**
Fregoli syndrome	Different people are the **same persecutor in disguise**
Cotard syndrome	**Nihilistic delusion** – "I am dead/ my organs are rotting" (severe depression)
Koro	Culture-bound – fear of penis retracting into abdomen (NE India, SE Asia)
Dhat syndrome	Culture-bound (India) – anxiety about **loss of semen**

PYQ style

- Schizophrenia term coined by – **Bleuler**; dementia praecox – **Kraepelin**.
- Schizophrenia means – **splitting of mental functions (not split personality)**.
- Bleuler's 4 A's – **association, affect, ambivalence, autism**.
- First-rank symptoms – **Kurt Schneider**; example – **thought broadcasting, voices commenting**.
- Most common hallucination – **auditory**.
- Dopamine hypothesis – **excess dopamine (mesolimbic)**.
- Negative symptoms – **alogia, avolition, anhedonia, flat affect, asociality**.
- Most common type – **paranoid**; worst prognosis – **hebephrenic**; best – **catatonic**.
- Waxy flexibility – **catatonic schizophrenia**.
- Poor prognostic factor – **insidious onset, negative symptoms, male, early onset**.
- DSM-5 duration – **6 months**.
- Treatment-resistant – **clozapine**.
- Hallucinating patient – **present reality, don't argue**.
- Delusional patient – **don't argue or agree; focus on feelings**.
- Patient refusing food fearing poison – **sealed food / let patient choose**.
- Shared psychotic disorder – **folie à deux**.
- Othello syndrome – **delusional jealousy**; de Clérambault – **erotomania**.
- "I am dead" – **Cotard syndrome**; impostor – **Capgras**.
- Postpartum psychosis – **psychiatric emergency (infanticide risk)**.
- Dhat syndrome – **semen loss anxiety (India)**.

6 Mood (Affective) Disorders & Suicide

6.1 Depression (Major Depressive Disorder)

- **Most common psychiatric disorder**; lifetime prevalence ~15 %; **women 2 : 1**; leading cause of disability worldwide (WHO).
- **Aetiology** – **monoamine hypothesis (↓ serotonin, norepinephrine, dopamine)**, **HPA axis hyperactivity (↑ cortisol)**, genetic, **loss/ stress (life events)**, **Beck's cognitive triad**, **learned helplessness (Seligman)**, personality, medical illness (hypothyroidism, stroke, cancer), drugs (**reserpine, steroids, beta-blockers, OCPs**).
- **Diagnosis – ≥ 2 weeks**, most of the day, nearly every day; **core symptoms – depressed mood, anhedonia (loss of interest/ pleasure), fatigue/ ↓ energy**.
- **Other symptoms (mnemonic "SIG E CAPS")** – **S**leep disturbance (**early morning awakening – typical**), **I**nterest loss, **G**uilt/ worthlessness, **E**nergy ↓, **C**oncentration ↓, **A**ppetite/ weight change, **P**sychomotor retardation/ agitation, **S**uicidal thoughts.
- **Biological (somatic/ melancholic) features** – **early morning awakening (2 hrs early)**, **diurnal variation (worse in the morning)**, loss of appetite & weight, psychomotor retardation, loss of libido, marked anhedonia.
- **Severity** – mild, moderate, **severe without/ with psychotic symptoms** (**mood-congruent delusions – guilt, poverty, nihilism**; Cotard).
- **Masked depression** – presents with somatic complaints (common in India – aches, pains).
- **Other forms** – **dysthymia (persistent depressive disorder) – mild depression ≥ 2 years**; **seasonal affective disorder (winter – light therapy)**; **postpartum depression** (2 weeks – 1 yr; vs **postpartum blues – day 3–5, self-limiting**); **atypical depression** (hypersomnia, hyperphagia, mood reactivity – MAOIs); **premenstrual dysphoric disorder**; grief vs depression (grief – self-esteem preserved).

Treatment

- **Antidepressants** (**SSRIs first-line**), **ECT** (severe, suicidal, psychotic, catatonic, pregnancy), **psychotherapy – CBT, IPT**, rTMS, light therapy; **treat for 6–12 months after remission**.

Nursing care of depressed patient

- **Safety first – assess suicide risk** (ask directly), **suicide precautions**, close observation (**especially when improving – energy ↑**).
- **Nutrition & fluids** (weigh, small frequent meals, favourite foods), **sleep hygiene**, **elimination (constipation)**, **personal hygiene – assist ADLs**.
- **Spend time with patient**, **accept silence**, **simple, short conversation**, **don't be overly cheerful** ("cheer up" – non-therapeutic).
- **Simple, structured, achievable tasks** → success → self-esteem; **positive reinforcement**; **avoid complex decisions** (impaired concentration).
- **Encourage expression of feelings (anger, guilt)**, cognitive restructuring (challenge negative thoughts), gradually increase activities & social interaction.
- **Medication** – delayed onset (2–4 weeks), compliance, side effects; **dispense limited quantity** (overdose risk).

6.2 Mania & Bipolar Disorder

- **Bipolar affective disorder (BPAD)** – recurrent episodes of **mania/ hypomania & depression**; **lifetime ~1 %**, **equal sex**; **onset ~20s**; **strong genetic component**.
- **Bipolar I** – **≥ 1 manic episode** (± depression); **Bipolar II** – **hypomania + major depression** (no full mania); **cyclothymia** – **≥ 2 yrs of mild mood swings** (hypomanic & depressive symptoms); **rapid cycling – ≥ 4 episodes/ year** (valproate).
- **Mania – ≥ 1 week** (any duration if hospitalised); **hypomania – ≥ 4 days**, no psychotic features, no marked impairment.

Features of mania ("DIG FAST")

- **D**istractibility, **I**ndiscretion/ **I**mpulsivity (reckless spending, sexual indiscretion), **G**randiosity (inflated self-esteem, delusions of grandeur), **F**light of ideas, **A**ctivity ↑ (psychomotor agitation, goal-directed), **S**leep need ↓ (**decreased need for sleep**), **T**alkativeness (**pressure of speech**).
- **Mood** – **elated/ euphoric**, **irritable** (when thwarted), **labile**; **infectious gaiety**; **clang associations, punning**; **colourful/ bizarre dress, heavy make-up**; **poor judgement**, **lack of insight**; exhaustion, dehydration, weight loss.

Treatment

- **Acute mania** – **lithium/ valproate + antipsychotic** (olanzapine, haloperidol, risperidone), **BZD** (sedation), **ECT** (resistant/ severe).
- **Maintenance (prophylaxis)** – **lithium (gold standard)**, valproate, lamotrigine (bipolar depression), quetiapine.
- **Antidepressant alone** may precipitate mania ("**switch**").

Nursing care of manic patient

- **Safety** – of patient & others; **↓ environmental stimuli** (**quiet, single room**, dim lights, avoid group/ competitive activities), **remove hazardous objects**.
- **Firm, calm, consistent approach**; **set limits** on behaviour (clearly, consistently by all staff); **avoid arguing/ power struggles**; don't laugh at jokes/ encourage.
- **Short, simple, clear communication**; **redirect** energy into **non-competitive physical activities requiring gross motor movement (walking, cleaning, sweeping)** – **avoid competitive games**.
- **Nutrition** – **high-calorie, high-protein finger foods & fluids "on the go"** (can't sit to eat), monitor weight & I/O (**dehydration, exhaustion**).
- **Rest & sleep** – quiet environment, limit caffeine, sedation as ordered.
- **Hygiene & dress** – supervise.
- **Protect from impulsive acts** (spending, sexual), **manipulation** – consistent limits.
- **Lithium** – monitor levels & toxicity; teach.
- **Psychoeducation** – early warning signs, sleep regularity, compliance, avoid substances.

6.3 Suicide

- **Suicide** – intentional self-inflicted death; **attempted suicide/ deliberate self-harm**; **suicidal ideation**.
- **India (NCRB 2022)** – **~1.7 lakh suicides/ year** (rate ~12 per lakh); **hanging – most common method**, followed by **poisoning (pesticides/ insecticides)**; most common causes – **family problems & illness**;

daily wage earners – largest group; **Maharashtra** highest number.

- **WHO** – > 7 lakh deaths/ yr globally; **4th leading cause of death in 15–29 yrs**.
- **World Suicide Prevention Day – 10 September**; **National Suicide Prevention Strategy (Nov 2022) – reduce suicide mortality by 10 % by 2030**; **Tele-MANAS 14416**; **MHCA 2017 Sec 115 – decriminalises suicide attempt** (presumed severe stress; govt duty to care & rehabilitate) – overrides the old **IPC Section 309** (punishment for attempted suicide).

Risk factors

- **Previous suicide attempt (strongest predictor)**, **depression (most common associated illness – ~15 % of severely depressed die by suicide)**, **bipolar**, **schizophrenia**, **alcohol/ substance use**, **personality disorder (borderline)**, chronic painful illness, **hopelessness (key psychological factor)**.
- **Male sex (completed suicide 3 ×), female (attempts 3–4 ×)**; elderly & young adults; **single, divorced, widowed**, living alone, **unemployment, financial loss, recent loss/ bereavement**, family history of suicide, access to means (**pesticides, firearms**), **lack of social support**.
- **SAD PERSONS scale** – **S**ex (male), **A**ge (< 19 or > 45), **D**epression, **P**revious attempt, **E**thanol abuse, **R**ational thinking loss (psychosis), **S**ocial support lacking, **O**rganised plan, **N**o spouse, **S**ickness (chronic) – score ≥ 7 → hospitalise.

Warning signs (clues)

- **Talking about death/ suicide ("I won't be a problem for you much longer")**, **giving away possessions**, **making a will, saying goodbyes**, withdrawal, **sudden calmness/ improved mood after depression** (decision made), hopelessness, collecting pills/ buying rope/ pesticide, increased substance use, reckless behaviour.

Assessment

- **Ask directly & clearly** – "**Are you thinking of killing yourself?**" – **asking does NOT increase the risk** (myth); explore **plan (method, means, time, place – lethality & specificity)**, intent, previous attempts, protective factors.

Nursing management (suicide precautions)

- **Ensure safety – highest priority**: **constant one-to-one observation (within arm's length for high risk)**, **room near nursing station**, **remove all potentially harmful objects (sharps, belts, ropes, shoelaces, glass, cords, plastic bags, medications)**, check items brought by visitors, **unbreakable windows**, ligature-free bathroom, **check mouth after medication (hoarding)**, observe **closely during shift change, night & early morning, weekends** (high-risk times), **when mood improves suddenly**.
- **Therapeutic relationship** – listen non-judgementally, **convey hope**, explore feelings & reasons for living, **no-suicide contract (adjunct only, not a substitute for observation)**.
- **Treat underlying illness** (antidepressants, **ECT**), crisis intervention, problem solving, **mobilise support systems**, involve family.
- **Discharge** – safety plan, restricted access to means (pesticide storage locked – **community pesticide storage**), follow-up, helplines.
- **Gatekeeper training** (QPR – Question, Persuade, Refer).

PYQ style

- Most common psychiatric disorder – **depression**.
- Depression duration criterion – **≥ 2 weeks**.
- Typical sleep disturbance in depression – **early morning awakening**.
- Diurnal variation – **worse in the morning**.
- Cognitive triad – **Beck**; learned helplessness – **Seligman**.
- Mild depression ≥ 2 yrs – **dysthymia**; mood swings ≥ 2 yrs – **cyclothymia**.
- Mania duration – **≥ 1 week**; hypomania – **≥ 4 days**.
- Bipolar II – **hypomania + depression**.
- Mania features – **DIG FAST (distractibility, grandiosity, flight of ideas, ↓ sleep, pressure of speech)**.
- Prophylaxis of bipolar – **lithium**.
- Manic patient activity – **non-competitive, gross motor (walking, sweeping)**.
- Manic patient diet – **high-calorie finger foods**.
- Strongest predictor of suicide – **previous attempt**.
- Most common psychiatric illness in suicide – **depression**.
- Most common suicide method in India – **hanging**; then **poisoning**.
- Completed suicide – **males**; attempts – **females**.
- Suicide risk scale – **SAD PERSONS**.
- High suicide risk period – **when depression begins to lift**.
- Asking about suicide – **does not increase risk**.
- Priority nursing care – **safety, constant observation, remove harmful objects**.
- Suicide attempt decriminalised – **MHCA 2017, Section 115**.
- World Suicide Prevention Day – **10 September**.

7 Anxiety, Obsessive-Compulsive, Stress-Related, Dissociative & Somatoform Disorders

7.1 Anxiety – Basics

- **Anxiety** – vague, diffuse feeling of apprehension/ uneasiness in response to an **unknown/ internal threat** (vs **fear – response to a known external danger**).
- **Physical symptoms (sympathetic)** – palpitation, tachycardia, sweating, tremor, dry mouth, **hyperventilation**, butterflies, frequent micturition, diarrhoea, dizziness.
- **Levels (Peplau)** – **mild** (alert, learning ↑), **moderate** (narrowed perceptual field, selective inattention), **severe** (very narrow, focus on details, can't learn), **panic** (disorganised, terror, loss of control, may harm self).
- **Neurotransmitters** – **↓ GABA**, ↑ norepinephrine, serotonin dysregulation.

7.2 Types of Anxiety Disorders

Disorder	Key features	Treatment
Generalised Anxiety Disorder (GAD)	**Excessive, uncontrollable worry about everyday matters**, **most days for ≥ 6 months**; restlessness, fatigue, muscle tension, poor concentration, irritability, insomnia	**SSRIs/ SNRIs (first-line)**, **buspirone**, short-term BZD, **CBT, relaxation**
Panic disorder	**Recurrent unexpected panic attacks** – sudden intense fear **peaking within 10 min** – palpitations, chest pain, **choking, sweating, fear of dying/ going crazy**, derealisation; **anticipatory anxiety**; ± agoraphobia	**SSRIs**, BZD (alprazolam/ clonazepam), **CBT**; during attack – **stay with patient, calm, slow breathing (paper bag for hyperventilation – rarely)**
Phobic disorders	**Persistent irrational fear** of specific object/ situation → **avoidance**; insight present	**Systematic desensitisation (Wolpe)**, **exposure therapy, flooding**, SSRIs; propranolol (performance anxiety)
– **Agoraphobia**	Fear of **open/ public places, crowds, situations where escape is difficult** (most disabling)	–
– **Social phobia (social anxiety)**	Fear of **scrutiny/ embarrassment** in social situations (public speaking)	SSRIs, CBT
– **Specific phobia**	Single object/ situation – **most common**	Exposure

Phobia	Fear of
Acrophobia	**Heights**
Claustrophobia	**Closed spaces**
Agoraphobia	Open spaces/ crowds
Hydrophobia	Water (**rabies**)
Nyctophobia	Darkness

Phobia	Fear of
Xenophobia	Strangers/ foreigners
Zoophobia / cynophobia	Animals / dogs
Ailurophobia	Cats
Arachnophobia	Spiders
Haematophobia	Blood
Thanatophobia	Death
Mysophobia	Dirt/ germs
Pyrophobia	Fire
Algophobia	Pain
Nosophobia	Disease
Trypanophobia	Injections/ needles
Glossophobia	Public speaking
Aerophobia	Flying
Ophidiophobia	Snakes

7.3 Obsessive-Compulsive Disorder (OCD)

- **Obsessions** – **recurrent, persistent, intrusive, unwanted thoughts, images or impulses**, recognised as **one's own** (product of own mind) & **irrational** (ego-dystonic), patient **tries to resist** → anxiety.
- **Compulsions** – **repetitive behaviours or mental acts** performed to **reduce anxiety** caused by obsessions (not pleasurable).
- **Common** – **contamination → washing (most common)**, doubt → **checking**, symmetry → ordering/ arranging, counting, **intrusive aggressive/ sexual/ religious thoughts**, hoarding.
- **Onset** – adolescence/ early adulthood; **chronic**; **insight usually present**; **serotonin dysfunction**; **basal ganglia/ orbitofrontal circuit**; associated with **depression**, Tourette's.
- **Defence mechanisms** – **undoing, reaction formation, isolation**.
- **Treatment** – **SSRIs in high doses (fluoxetine, fluvoxamine, sertraline)** / **clomipramine** (TCA – most serotonergic); **Exposure & Response Prevention (ERP) – psychotherapy of choice**; deep brain stimulation (refractory).
- **Y-BOCS** – severity scale.
- **Nursing** – **don't interrupt or stop the ritual abruptly** (↑ anxiety) – **allow time for rituals initially**, then **gradually limit/ set limits** with patient's agreement; structured schedule; **meet physical needs (skin care – dermatitis from washing)**; support during ERP; positive reinforcement; reduce anxiety; involve family (avoid accommodating rituals).
- **OCD spectrum** – **body dysmorphic disorder** (imagined defect in appearance), **trichotillomania (hair pulling)**, **hoarding disorder**, excoriation (skin picking).

7.4 Stress-Related Disorders

Disorder	Features
Acute stress reaction / disorder	After **exceptional stressor**; symptoms within hours–days, **resolve within days–1 month**; daze, disorientation, anxiety, dissociation
Post-Traumatic Stress Disorder (PTSD)	After **life-threatening trauma** (war, rape, disaster, accident); **symptoms > 1 month** (onset may be delayed up to 6 months); **re-experiencing (flashbacks, nightmares, intrusive memories)**, **avoidance** of reminders, **hyperarousal (hypervigilance, exaggerated startle, insomnia, irritability)**, negative mood, numbness, **survivor guilt**; Rx – **trauma-focused CBT, EMDR**, **SSRIs (sertraline, paroxetine)**, prazosin (nightmares)
Adjustment disorder	Emotional/ behavioural symptoms **within 1 month (DSM – 3 months) of identifiable stressor** (divorce, job loss, migration); resolves within **6 months**; supportive therapy
Grief / bereavement	Normal response to loss; **Kübler-Ross stages – Denial → Anger → Bargaining → Depression → Acceptance (DABDA)**; **prolonged grief disorder** (ICD-11 – > 6 months)

- **Stress (Hans Selye)** – **General Adaptation Syndrome (GAS)** – **alarm reaction → resistance → exhaustion**; **eustress** (positive) vs **distress**; **Holmes & Rahe Social Readjustment Rating Scale** (life events – **death of spouse = 100**, highest).
- **Coping** – problem-focused & emotion-focused (Lazarus).

7.5 Dissociative (Conversion) Disorders

- **Dissociation** – **disruption of normal integration of consciousness, memory, identity, perception or motor control**, usually after **psychological stress/ conflict** (defence mechanism).

Disorder	Features
Dissociative (conversion) motor/ sensory disorder (functional neurological symptom disorder)	**Loss/ alteration of motor or sensory function** (paralysis, **pseudoseizures (dissociative convulsions)**, aphonia, blindness, anaesthesia) **without organic cause**, following stress; **"la belle indifférence" (inappropriate lack of concern)**; **primary gain** (relief of anxiety) & **secondary gain** (attention, escape from responsibility); common in young females, low SES; Rx – **reassurance, suggestion, psychotherapy, avoid reinforcing secondary gain**, focus on stressors
Dissociative amnesia	**Inability to recall important personal (usually traumatic) information**; **localised** – most common
Dissociative fugue	**Sudden, unexpected travel away from home** with **amnesia for past & identity confusion/ new identity**
Dissociative identity disorder (multiple personality)	**≥ 2 distinct personality states** taking control; history of **childhood abuse**
Depersonalisation/ derealisation disorder	Persistent feelings of unreality of self/ surroundings; reality testing intact
Possession/ trance disorder	Culture-related (common in India) – temporary loss of identity, feels "possessed by spirit/ deity"

Pseudoseizure vs true seizure

	Dissociative (pseudo) seizure	Epileptic seizure
Situation	In presence of others, emotional trigger	Any time, even sleep
Injury, tongue bite, incontinence	**Rare**	Common
Movements	Bizarre, asynchronous, **pelvic thrusting**, eyes closed tightly (resists opening)	Stereotyped, eyes open
Post-ictal confusion	Absent	Present
Serum prolactin	**Normal**	**↑ (within 20 min)**
EEG	Normal	Abnormal

7.6 Somatic Symptom (Somatoform) Disorders

Disorder	Features
Somatisation disorder / somatic symptom disorder	**Multiple, recurrent physical complaints** (pain, GI, sexual, pseudoneurological) **for years, no organic basis**; doctor shopping; females
Hypochondriasis (illness anxiety disorder)	**Preoccupation with fear of having a serious disease** (e.g. cancer) despite **negative investigations & reassurance**
Persistent somatoform pain disorder	Chronic pain without adequate physical cause
Body dysmorphic disorder	Imagined/ slight defect in appearance (OCD spectrum)
Factitious disorder (Munchausen syndrome)	**Intentional production of symptoms to assume sick role** (no external reward); **by proxy** – caregiver induces illness in child
Malingering	**Intentional** faking for **external gain** (money, avoid duty, legal) – **not a mental disorder**

- **Nursing (somatoform)** – **acknowledge symptoms are real to patient**, avoid repeated unnecessary investigations, **don't reinforce sick role**, **focus on feelings & stressors** (not symptoms), teach coping/ relaxation, gradual return to responsibilities, consistent single caregiver.

7.7 General Nursing Care in Anxiety

- **Stay with patient** during severe anxiety/ panic; **calm, quiet environment**; **short, simple sentences**; **reduce stimuli**; **help identify & verbalise feelings**; **deep breathing, relaxation, distraction**; **physical activity**; recognise triggers; **avoid caffeine**; teach **coping skills, problem solving**; medication (BZD short-term – dependence).

PYQ style

- Anxiety vs fear – fear has **known external object**.
- Excessive worry ≥ 6 months – **GAD**.
- Recurrent sudden attacks with fear of dying – **panic disorder**.
- Fear of open places/ crowds – **agoraphobia**; heights – **acrophobia**; closed spaces – **claustrophobia**.
- Hydrophobia – **water (rabies)**; nyctophobia – **darkness**; xenophobia – **strangers**.
- Phobia treatment – **systematic desensitisation**.
- Recurrent intrusive thoughts recognised as own – **obsessions**.
- Most common obsession/ compulsion – **contamination → washing**.
- OCD psychotherapy – **ERP**; drug – **SSRIs / clomipramine**.
- Don't stop rituals abruptly – **↑ anxiety**.
- PTSD duration – **> 1 month**; key features – **flashbacks, avoidance, hyperarousal**.
- Kübler-Ross stages – **denial, anger, bargaining, depression, acceptance**.
- General Adaptation Syndrome – **Hans Selye (alarm, resistance, exhaustion)**.
- Highest life-event stress score – **death of spouse**.
- La belle indifférence – **conversion (dissociative) disorder**.
- Sudden travel with amnesia – **dissociative fugue**.
- Pseudoseizure – **normal prolactin, no post-ictal confusion**.
- Preoccupation with having serious disease – **hypochondriasis**.
- Intentional symptoms for sick role – **factitious (Munchausen)**; for external gain – **malingering**.

8 Substance Use Disorders

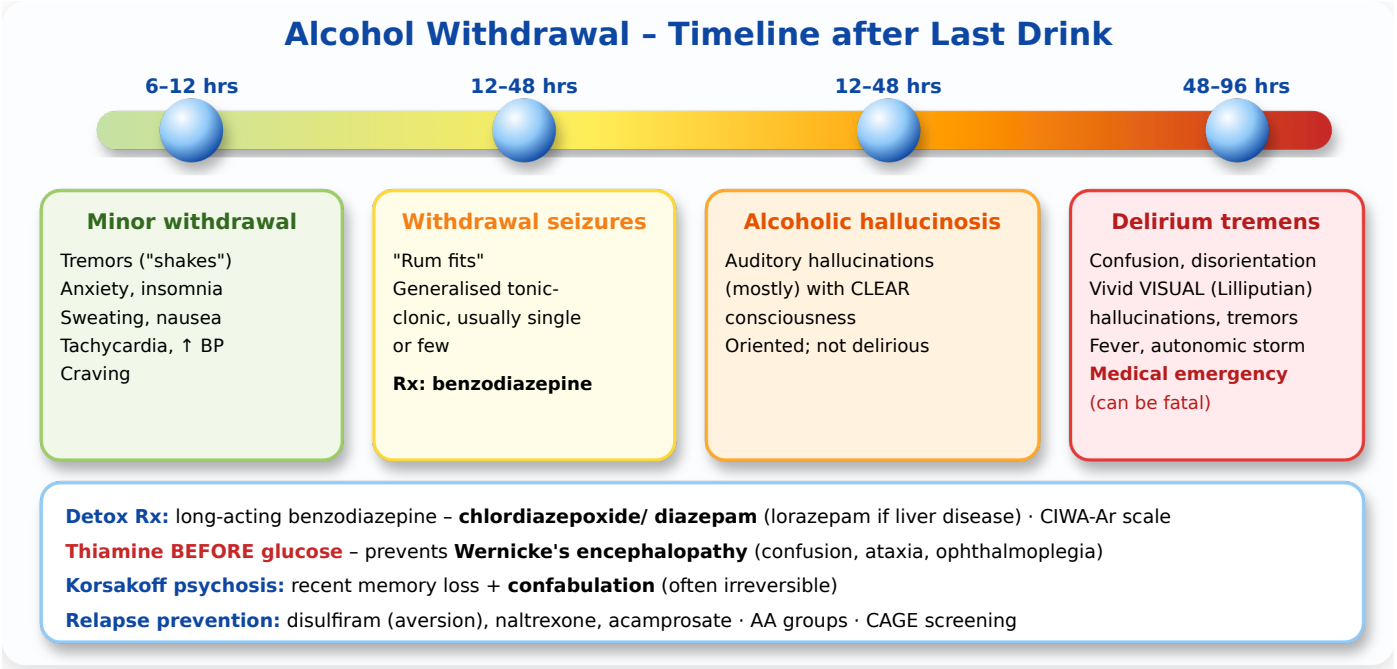
8.1 Terminology

Term	Meaning
Substance abuse / harmful use	Pattern of use causing **physical or mental harm**
Dependence (addiction)	Cluster – **craving (compulsion)**, **loss of control**, **tolerance**, **withdrawal**, **neglect of other interests**, **persistent use despite harm** (≥ 3 in 12 months – ICD-10)
Tolerance	**Need for increased amounts** for same effect / diminished effect with same dose
Withdrawal	Characteristic symptoms on stopping/ reducing (usually opposite of drug effect)
Cross-tolerance	Tolerance to one drug extends to similar drugs (alcohol ↔ BZD – used in detox)
Intoxication	Reversible state after recent use
Relapse	Return to use after abstinence
Psychological dependence	Craving, emotional need; **physical dependence** – withdrawal syndrome

- **National Survey (Magnitude of Substance Use, 2019, AIIMS)** – **alcohol – most commonly used (~14.6 % of 10–75 yrs)**, then **cannabis (~2.8 %)** & **opioids (~2.1 %)**; **tobacco** – GATS-2: 28.6 % adults.
- **Aetiology** – genetic (alcohol), peer pressure, availability, stress, personality, family dysfunction, **reward pathway – dopamine (mesolimbic, nucleus accumbens)**.

8.2 Alcohol

- **CNS depressant**; metabolised in liver – **alcohol dehydrogenase → acetaldehyde → aldehyde dehydrogenase → acetate**; **1 standard drink ~10 g alcohol (India)**.
- **Blood alcohol** – **legal driving limit India 30 mg/100 ml (0.03 %)**; > 400 mg% may be fatal.
- **Screening** – **CAGE** (**C**ut down?, **A**nnoyed by criticism?, **G**uilty?, **E**ye-opener? – **≥ 2 = significant**), **AUDIT** (WHO, 10 items), lab – **↑ GGT (most sensitive)**, ↑ MCV, ↑ AST:ALT > 2, **CDT**.
- **Complications** – **GI** (gastritis, pancreatitis, **fatty liver → alcoholic hepatitis → cirrhosis**, oesophageal varices), **CNS** (**Wernicke-Korsakoff, peripheral neuropathy**, cerebellar degeneration, dementia), **cardiomyopathy**, HTN, **fetal alcohol syndrome**, nutritional deficiencies (**thiamine**, folate), **accidents, violence, suicide**, **Othello syndrome (morbid jealousy)**, social & occupational problems.



Alcohol withdrawal timeline – tremors, seizures, hallucinosis & delirium tremens

Alcohol withdrawal

Stage	Time after last drink	Features
Uncomplicated withdrawal	**6–12 hrs** (peak 24–48)	**Tremors (earliest – "shakes")**, anxiety, insomnia, sweating, nausea, tachycardia, ↑ BP, craving
Withdrawal seizures ("rum fits")	**12–48 hrs**	Generalised tonic-clonic
Alcoholic hallucinosis	12–48 hrs	**Auditory hallucinations** in **clear consciousness**
Delirium tremens (DT)	**48–72 hrs (up to 96)**	**Medical emergency** – **clouded consciousness, disorientation, vivid visual (Lilliputian – small people/ animals) & tactile hallucinations, gross tremor, fever, autonomic hyperactivity (tachycardia, sweating, ↑ BP)**, agitation; mortality ~5 % untreated

Treatment

- **Detoxification** – **benzodiazepines (drug of choice)** – **chlordiazepoxide/ diazepam** (long-acting – loading/ tapering), **lorazepam/ oxazepam in liver disease & elderly**; **CIWA-Ar** guided; **IV thiamine 100 mg before glucose** (prevents Wernicke's), multivitamins, fluids & electrolytes (Mg, K), nutrition.
- **DT nursing** – **quiet, well-lit room** (reduce misperceptions), **constant observation**, **re-orientation**, **safety (side rails, avoid restraints if possible)**, vital signs every 1–2 hrs, **fluid & electrolyte balance**, fever control, seizure precautions, high-calorie diet.
- **Wernicke's encephalopathy** – **thiamine (B1) deficiency** – **triad: global confusion, ataxia, ophthalmoplegia (nystagmus, lateral rectus palsy)** – reversible with **parenteral thiamine**.
- **Korsakoff's psychosis/ syndrome** – **anterograde amnesia (recent memory loss), disorientation & confabulation** – largely **irreversible**; mammillary bodies.

- **Relapse prevention (anti-craving/ deterrent)** –
- **Disulfiram (Antabuse)** – inhibits aldehyde dehydrogenase → **acetaldehyde accumulates → disulfiram-ethanol reaction (flushing, throbbing headache, nausea, vomiting, palpitation, hypotension, breathlessness)**; **start after ≥ 12–24 hrs of abstinence**, **informed consent – never secretly**; avoid alcohol in **cough syrups, mouthwash, aftershave, vinegar sauces**.
- **Naltrexone** (opioid antagonist – ↓ craving & reward), **acamprosate** (glutamate – ↓ craving; safe in liver disease), baclofen, topiramate.
- **Psychosocial** – **motivational interviewing (stages of change)**, **CBT, relapse prevention**, **Alcoholics Anonymous (AA) – 12-step programme (1935, Bill W. & Dr Bob)**, **Al-Anon (family), Alateen (teenage children)**, family therapy, aversion therapy, **de-addiction centres**.

8.3 Other Substances

Substance	Intoxication	Withdrawal	Specific points
Opioids (heroin/ "brown sugar", morphine, codeine cough syrup, tramadol, opium, smack)	Euphoria, drowsiness, **pin-point pupils (miosis)**, **respiratory depression**, constipation; **overdose triad – coma, pin-point pupils, respiratory depression**	**Flu-like – yawning, lacrimation, rhinorrhoea, dilated pupils (mydriasis), piloerection ("cold turkey"), muscle aches, abdominal cramps, diarrhoea, insomnia, craving**; **not life-threatening**	**Overdose – naloxone**; detox – **buprenorphine, methadone, clonidine**; **OST (opioid substitution therapy) – buprenorphine/ methadone**; **naltrexone** (maintenance); IV use – **HIV, HCV**, abscess
Cannabis (bhang, ganja, charas/ hashish; **THC**)	Euphoria, **↑ appetite ("munchies")**, **conjunctival injection (red eyes)**, dry mouth, tachycardia, time distortion, anxiety/ panic	Irritability, insomnia, ↓ appetite (mild)	**Amotivational syndrome**, **cannabis-induced psychosis**, ↑ schizophrenia risk; "flashbacks"
Tobacco/ nicotine	Stimulant & relaxant	Irritability, craving, ↑ appetite, poor concentration	**Most common substance used (incl. smokeless)**; **NRT, varenicline, bupropion**; **5 A's (Ask, Advise, Assess, Assist, Arrange)**; quitline **1800-112-356**
Cocaine / amphetamines (stimulants)	Euphoria, **↑ energy, dilated pupils**, tachycardia, HTN, **paranoid psychosis**, **formication ("cocaine bugs")**, MI, stroke	**"Crash" – depression, hypersomnia, ↑ appetite, suicidal**	Nasal septum perforation (cocaine)
Sedative-hypnotics (BZD, barbiturates)	Like alcohol – slurred speech, ataxia	**Like alcohol – seizures, delirium (can be fatal) → gradual tapering**	Overdose BZD – **flumazenil**
Inhalants/ volatile solvents (glue, **whitener, petrol**, paint thinner)	Euphoria, dizziness, slurred speech	–	**Street children, adolescents**; sudden sniffing death (arrhythmia), brain/ liver/ kidney damage

Substance	Intoxication	Withdrawal	Specific points
Hallucinogens (LSD, mescaline, psilocybin)	Perceptual distortions, **synaesthesia**, "bad trip", flashbacks	Minimal	–
Caffeine	Restlessness	Headache, fatigue	–

8.4 Nursing Care & Programmes

- **Assessment** – history (substance, amount, route, duration, last use), withdrawal signs, complications, motivation (**stages of change**), family.
- **Detox phase** – safety, **vital signs**, withdrawal monitoring (CIWA/ COWS), nutrition, hydration, **seizure precautions**, sleep, medications.
- **Rehabilitation** – **therapeutic relationship (non-judgemental)**, **confront denial** gently, **motivational interviewing**, **coping skills, relapse prevention**, **life skills, vocational training**, **self-help groups (AA, NA)**, family involvement, **avoid enabling** (codependency).
- **Common defence mechanisms** – **denial (most common)**, rationalisation, projection, minimisation.
- **Laws & programmes** – **NDPS Act 1985 (Narcotic Drugs & Psychotropic Substances)** – addicts may be sent for treatment (Sec 64A immunity if seeking treatment); **Nasha Mukt Bharat Abhiyaan (15 Aug 2020)**; **National Action Plan for Drug Demand Reduction (NAPDDR)**; **de-addiction centres (Integrated Rehabilitation Centres for Addicts – IRCA)**; **Drug Treatment Clinics**; **International Day against Drug Abuse & Illicit Trafficking – 26 June**; **Gujarat – prohibition state (Bombay Prohibition Act 1949)** – health permits for alcohol.

PYQ style

- Most commonly used substance (excluding tobacco) in India – **alcohol**.
- Need for increased dose – **tolerance**.
- CAGE – **≥ 2 positive = alcohol problem**; most sensitive lab marker – **GGT**.
- Earliest alcohol withdrawal sign – **tremors (6–12 hrs)**.
- Rum fits – **12–48 hrs**.
- Delirium tremens onset – **48–72 hrs**; hallucinations – **visual (Lilliputian)**.
- Alcohol detox drug – **chlordiazepoxide/ diazepam**; in liver disease – **lorazepam**.
- Thiamine given **before glucose** – prevent **Wernicke's encephalopathy**.
- Wernicke's triad – **confusion, ataxia, ophthalmoplegia**.
- Korsakoff – **amnesia + confabulation**.
- Disulfiram – **aldehyde dehydrogenase inhibitor**; reaction with **alcohol**.
- Anti-craving – **naltrexone, acamprosate**.
- Alcoholics Anonymous – **12-step programme**; family group – **Al-Anon**.
- Opioid intoxication – **pin-point pupils, respiratory depression**; antidote – **naloxone**.
- Opioid withdrawal – **dilated pupils, yawning, lacrimation, piloerection**.
- OST – **buprenorphine/ methadone**.
- Cannabis – **red eyes, ↑ appetite, amotivational syndrome**.
- Cocaine bugs – **formication (tactile hallucination)**.
- Most common defence in addiction – **denial**.
- NDPS Act – **1985**; anti-drug day – **26 June**.

9 Personality, Eating, Sleep & Sexual Disorders

9.1 Personality Disorders (PD)

- **Personality** – enduring pattern of thinking, feeling & behaving.
- **Personality disorder** – **enduring, inflexible, maladaptive pattern** of inner experience & behaviour, deviating markedly from culture, **pervasive**, **onset in adolescence/ early adulthood**, causing distress/ impairment; usually **ego-syntonic** (patient doesn't see it as a problem) – **poor insight, difficult to treat**.

Cluster	Type	Key features
A – "Odd/ eccentric" (MAD)	**Paranoid**	**Pervasive distrust & suspiciousness**, grudges, misinterprets benign remarks
	Schizoid	**Detached, loner, emotionally cold**, indifferent to praise/ criticism, no desire for relationships
	Schizotypal	**Odd beliefs, magical thinking, eccentric behaviour/ appearance**, ideas of reference (closest to schizophrenia)
B – "Dramatic/ emotional/ erratic" (BAD)	**Antisocial (dissocial)**	**Disregard for others' rights, lying, cheating, aggression, criminal acts, impulsivity, irresponsibility, LACK OF GUILT/ remorse**; **conduct disorder before 15 yrs**; diagnosed **≥ 18 yrs**; males
	Borderline (emotionally unstable)	**Unstable relationships, self-image & mood**; **fear of abandonment**, **impulsivity**, **recurrent self-harm (wrist cutting)/ suicidal behaviour**, **chronic emptiness**, intense anger, **splitting (idealisation ↔ devaluation)**, **manipulation**, transient paranoia; females
	Histrionic	**Attention-seeking, dramatic, seductive**, shallow emotions, suggestible
	Narcissistic	**Grandiose, need for admiration, lack of empathy**, entitlement, exploitative
C – "Anxious/ fearful" (SAD)	**Avoidant (anxious)**	**Social inhibition, feels inadequate, hypersensitive to rejection**, wants relationships but avoids
	Dependent	**Excessive need to be cared for, submissive, clinging**, can't make decisions alone
	Obsessive-compulsive (anankastic)	**Perfectionism, orderliness, rigidity, preoccupation with rules & details**, stubborn, miserly (ego-syntonic – vs OCD ego-dystonic)

- **Treatment** – psychotherapy (**Dialectical Behaviour Therapy – DBT – Marsha Linehan – for borderline PD**), CBT, group therapy; medication for symptoms (SSRIs, mood stabilisers, low-dose antipsychotics).

Nursing care

- **Borderline** – **safety (self-harm precautions)**, **consistent limit setting by all staff** (prevents **splitting** – staff communicate & present unified approach), **clear boundaries**, avoid special favours,

teach emotion regulation & coping, **don't reinforce manipulation**, help verbalise feelings, structured routine, DBT skills.

- **Antisocial** – **firm, consistent limits & consequences**, **don't argue/ get manipulated**, **hold patient responsible** for behaviour, protect others, teach anger management, positive reinforcement for appropriate behaviour.
- **Manipulative behaviour** – recognise, set & enforce limits consistently, **all staff aware of care plan**, avoid power struggles, don't accept gifts/ flattery, **discuss behaviour directly**.
- **Paranoid** – **honest, matter-of-fact**, explain actions, avoid humour/ whispering, respect space.
- **Dependent** – encourage decision making & self-care, praise independence.

9.2 Eating Disorders

	Anorexia nervosa	Bulimia nervosa
Core	**Deliberate weight loss; intense fear of gaining weight; distorted body image** (sees self fat)	**Recurrent binge eating** (large amount, loss of control) **+ compensatory behaviour** (**self-induced vomiting, laxatives, diuretics, fasting, excessive exercise**) – **at least once/ week × 3 months**
Weight	**Significantly low – BMI < 17.5 (ICD-10)/ < 18.5**	**Usually normal/ near normal**
Age/ sex	**Adolescent girls (13–20 yrs)**, middle/ high SES, perfectionists	Late adolescent/ young women
Physical signs	**Amenorrhoea (ICD-10), lanugo hair, bradycardia, hypotension, hypothermia**, dry skin, constipation, osteoporosis, ↓ T3, **hypokalaemia** (if purging), oedema	**Russell's sign (calluses on knuckles)**, **dental enamel erosion**, **parotid enlargement**, **hypokalaemic metabolic alkalosis**, oesophagitis/ Mallory-Weiss tear
Mortality	**Highest of all psychiatric disorders** (~5–10 % – cardiac arrhythmia, suicide)	Lower
Treatment	**Nutritional rehabilitation (gradual – avoid refeeding syndrome: ↓ phosphate, K, Mg)**, **target weight gain 0.5–1 kg/week (inpatient)**, **family-based therapy (Maudsley – adolescents)**, CBT; hospitalise if BMI very low, electrolyte imbalance, bradycardia	**CBT (first-line)**, **SSRIs (fluoxetine – high dose)**, nutrition counselling

- **Nursing (anorexia)** – **weigh daily (same time, same scale, after voiding, gown only, back to scale)**, **supervise meals & 1–2 hrs after meals (bathroom observation – prevent vomiting)**, **calorie count & I/O**, **contract for weight gain with privileges (behaviour modification)**, **don't discuss food/ weight excessively or argue about body image**, focus on feelings/ self-esteem, monitor electrolytes & ECG, **refeeding syndrome watch**, limit exercise.
- **Binge eating disorder** – binges without compensatory behaviour → obesity. **Pica** (non-food), **rumination disorder**.

9.3 Sleep Disorders

- **Normal sleep** – **NREM (stages N1, N2, N3 – deep/ slow-wave)** & **REM (dreaming, muscle atonia, rapid eye movements)**; cycles ~**90 min**, 4–6 cycles/ night; **REM ~20–25 %** of adult sleep; **newborn ~50 % REM**.

Disorder	Features
Insomnia	**Most common**; difficulty initiating/ maintaining sleep or early awakening ≥ 3 nights/ week × 3 months; Rx – **sleep hygiene, CBT-I (first-line)**, short-term hypnotics (zolpidem, BZD)
Hypersomnia	Excessive sleepiness
Narcolepsy	**Tetrad – excessive daytime sleepiness (sleep attacks), cataplexy (sudden loss of muscle tone with emotion – laughter), hypnagogic hallucinations, sleep paralysis**; **REM onset sleep**; ↓ hypocretin (orexin); Rx – **modafinil**, stimulants, sodium oxybate
Obstructive sleep apnoea	Snoring, apnoea, daytime sleepiness, obesity; **CPAP**
Sleepwalking (somnambulism), night terrors (pavor nocturnus)	**NREM stage 3 (deep sleep)**; children; **no memory** of event; ensure safety
Nightmares	**REM sleep**; vivid recall
REM sleep behaviour disorder	Acting out dreams (loss of REM atonia) – Parkinson's risk
Restless legs syndrome	Urge to move legs at rest/ evening; iron deficiency; dopamine agonists
Bruxism	Teeth grinding

- **Sleep hygiene** – **regular sleep-wake schedule**, **avoid daytime naps**, **avoid caffeine, nicotine, alcohol & heavy meals before bed**, exercise (not near bedtime), **bed only for sleep (stimulus control)**, quiet, dark, comfortable room, relaxation, **get up if unable to sleep in 20 min**, **avoid screens before bed**, warm milk.

9.4 Sexual Disorders

- **Sexual dysfunctions** – **erectile dysfunction (most common in males)**, **premature ejaculation**, ↓ desire (hypoactive sexual desire – most common in females), female arousal disorder, **anorgasmia**, **vaginismus** (involuntary spasm of vaginal muscles), **dyspareunia** (painful intercourse); **Masters & Johnson** – sexual response cycle (excitement, plateau, orgasm, resolution); **sensate focus therapy**; PDE-5 inhibitors (sildenafil) for ED; SSRIs (delay ejaculation).
- **Paraphilias (disorders of sexual preference)** – **fetishism** (non-living objects), **exhibitionism** (exposing genitals to strangers), **voyeurism** (watching others undress/ sexual acts), **frotteurism** (rubbing against non-consenting person), **paedophilia** (prepubertal children), **sexual sadism** (inflicting pain), **sexual masochism** (receiving pain), **transvestic fetishism** (cross-dressing for arousal), necrophilia, zoophilia.
- **Gender dysphoria (gender incongruence – ICD-11 moved out of mental disorders)** – distress due to incongruence between experienced gender & assigned sex; **Transgender Persons (Protection of Rights) Act 2019**.
- **Homosexuality** – **not a mental disorder** (removed DSM 1973); **Supreme Court of India decriminalised consensual same-sex relations – Sept 2018 (Navtej Singh Johar)**.
- **Dhat syndrome** (culture-bound, India).

PYQ style

- Cluster A – **odd/ eccentric (paranoid, schizoid, schizotypal)**.
- Cluster B – **dramatic (antisocial, borderline, histrionic, narcissistic)**.
- Cluster C – **anxious (avoidant, dependent, obsessive-compulsive)**.
- Lack of guilt, criminal behaviour – **antisocial PD**; must be **≥ 18 yrs** with conduct disorder before **15**.
- Self-harm, fear of abandonment, splitting – **borderline PD**; therapy – **DBT (Linehan)**.
- Splitting prevention – **consistent approach by all staff**.
- Attention seeking, dramatic – **histrionic**; grandiose, lacks empathy – **narcissistic**.
- Highest mortality psychiatric disorder – **anorexia nervosa**.
- Anorexia – **BMI < 17.5, amenorrhoea, lanugo, bradycardia, distorted body image**.
- Russell's sign, dental erosion – **bulimia nervosa**.
- Refeeding syndrome – **hypophosphataemia**.
- Supervise after meals – **prevent vomiting (eating disorders)**.
- Most common sleep disorder – **insomnia**; first-line treatment – **CBT-I/ sleep hygiene**.
- Cataplexy – **narcolepsy**; drug – **modafinil**.
- Sleepwalking & night terrors – **NREM stage 3**; nightmares – **REM**.
- Painful intercourse – **dyspareunia**; vaginal muscle spasm – **vaginismus**.
- Watching others undress – **voyeurism**; exposing – **exhibitionism**; objects – **fetishism**.

10 Neurocognitive (Organic) Disorders & Child–Adolescent Psychiatry

10.1 Delirium

D Delirium (acute confusional state)

Acute onset (hours–days), fluctuating disturbance of **attention & consciousness (awareness)** with **cognitive changes (disorientation, memory)** and **perceptual disturbances**, caused by an **underlying medical condition, drug intoxication or withdrawal**; usually **reversible**.

- **Causes ("I WATCH DEATH")** – **I**nfections (UTI, pneumonia – common in elderly), **W**ithdrawal (alcohol – DT, BZD), **A**cute metabolic (**hypoglycaemia, electrolyte imbalance – Na, Ca**, uraemia, hepatic failure), **T**rauma (head injury, post-op), **C**NS pathology (stroke, seizures), **H**ypoxia, **D**eficiencies (**thiamine, B12**), **E**ndocrine (thyroid), **A**cute vascular, **T**oxins/ drugs (**anticholinergics, opioids, BZD, steroids**), **H**eavy metals.
- **Features** – **clouding of consciousness**, **↓ attention**, **disorientation (time first)**, **visual hallucinations/ illusions**, agitation or **hypoactive (quiet) delirium – often missed**, **sleep-wake reversal**, **sundowning (worse in evening/ night)**, fluctuating course, lucid intervals.
- **Assessment** – **CAM (Confusion Assessment Method)**.
- **Management** – **identify & treat cause (most important)**; **low-dose haloperidol** for agitation; **avoid BZD** (except alcohol/ BZD withdrawal); **non-pharmacological**.
- **Nursing** – **well-lit room (night light), quiet, familiar objects, clocks/ calendars, frequent re-orientation**, **same staff**, **family presence**, **glasses & hearing aids**, safety (**fall prevention, low bed**), **avoid restraints**, hydration, nutrition, sleep promotion, early mobilisation.

10.2 Dementia (Major Neurocognitive Disorder)

- **Chronic, progressive, usually irreversible** decline in **memory & other cognitive functions** (language, judgement, executive function) with **clear consciousness**, interfering with daily life.
- **Types** – **Alzheimer's disease (most common ~60–70 %)**, **vascular dementia (2nd – stepwise deterioration, multi-infarct)**, **Lewy body dementia** (visual hallucinations, parkinsonism, fluctuations – **sensitive to antipsychotics**), **frontotemporal (Pick's disease – personality change, disinhibition, early)**, Parkinson's, HIV, alcohol-related; **reversible causes** – **hypothyroidism, B12 deficiency, normal pressure hydrocephalus (triad: dementia, gait disturbance, urinary incontinence – "wacky, wobbly, wet")**, depression (**pseudodementia**), subdural haematoma, neurosyphilis.
- **Alzheimer's** – **amyloid plaques & neurofibrillary tangles (tau)**, **↓ acetylcholine**, cortical atrophy (hippocampus); risk – **age (strongest)**, family history, **Down syndrome**, APOE-ε4; **4 A's – Amnesia (recent memory first), Aphasia, Apraxia, Agnosia**; stages – mild (forgetfulness) → moderate (confusion, wandering, sundowning) → severe (total dependence, incontinence).
- **Drugs** – **cholinesterase inhibitors – donepezil, rivastigmine (patch), galantamine** (mild–moderate); **memantine (NMDA antagonist – moderate–severe)**; behavioural symptoms – non-drug first; antipsychotics cautiously (↑ stroke/ death in elderly).

- **MMSE (Folstein, 30 points)** – **24–30 normal, 18–23 mild, < 17 severe**; **Mini-Cog, MoCA, clock drawing test**.
- **World Alzheimer's Day – 21 September**.

	Delirium	Dementia
Onset	**Acute (hours–days)**	**Insidious (months–years)**
Course	**Fluctuating**, worse at night	**Progressive, stable day-to-day**
Consciousness	**Clouded/ altered**	**Clear** (until late)
Attention	**Impaired (key feature)**	Normal (early)
Memory	Immediate & recent impaired	**Recent memory first**, later remote
Hallucinations	**Common (visual)**	Less common (except Lewy body)
Reversibility	**Usually reversible**	**Usually irreversible**
Duration	Days–weeks	Months–years

Nursing care of dementia

- **Safety** – remove hazards, **ID bracelet (wandering)**, locked exits, **fall prevention**, supervise smoking/ cooking/ medications.
- **Structured routine**, **consistent caregiver**, **simple one-step instructions**, **short sentences, calm tone, eye contact**, avoid arguing/ correcting false memories – **validation therapy** (accept feelings), **reality orientation** (early stages), **reminiscence therapy** (past memories – ↑ self-esteem).
- **ADL support** – dressing (simple clothes, Velcro), feeding (finger foods), toileting schedule (incontinence), hygiene.
- **Sundowning** – ↑ lighting in evening, limit naps, structured activities, avoid caffeine.
- **Agitation** – identify triggers (pain, hunger, toilet), distraction, calm environment.
- **Caregiver support** – **caregiver burden/ burnout**, **respite care**, support groups, education, legal/ financial planning (advance directive – MHCA).

10.3 Intellectual Disability (Intellectual Developmental Disorder)

- **IQ < 70** + **deficits in adaptive functioning** with **onset during developmental period (< 18 yrs)**; **IQ = MA/ CA × 100**.
- **Mild (50–69) – ~85 %, educable**; **moderate (35–49) – trainable**; **severe (20–34)**; **profound (< 20)**.
- **Causes** – **genetic (Down syndrome – most common chromosomal; Fragile X – most common inherited)**, metabolic (**PKU, congenital hypothyroidism – most common preventable**), prenatal (**rubella, alcohol, iodine deficiency**), perinatal (**birth asphyxia, prematurity, kernicterus**), postnatal (meningitis, encephalitis, head injury, lead, malnutrition), psychosocial deprivation.
- **Management** – **early stimulation**, **special education**, **vocational training**, behaviour modification (**token economy, operant conditioning**), **parental counselling**, treat comorbidities (epilepsy, behaviour problems – risperidone), **RPwD Act 2016**, **NIEPID (Secunderabad)**; **prevention** – iodised salt, rubella vaccination, folic acid, antenatal care, newborn screening (TSH, PKU).

10.4 Childhood Psychiatric Disorders

Disorder	Key features	Treatment/ nursing
Autism spectrum disorder (ASD)	Onset **< 3 yrs**; **deficits in social interaction & communication** (**poor eye contact, no social smile/ pointing, delayed speech, echolalia, pronoun reversal**), **restricted repetitive behaviours** (**hand flapping, rocking, insistence on sameness**), sensory issues; boys 4 : 1; **Kanner (1943)**; **M-CHAT** screening; **not caused by vaccines**	**Early intensive behavioural intervention (ABA)**, speech & occupational therapy, special education, **structured routine**, risperidone/ aripiprazole for irritability
ADHD	**Inattention, hyperactivity, impulsivity**; onset **before 12 yrs**, ≥ 2 settings, ≥ 6 months; boys	**Methylphenidate (DOC)**, atomoxetine; **behaviour therapy, parent training**; **structured environment, short tasks, front seat, one instruction at a time, positive reinforcement**, reduce distractions
Conduct disorder	**Repetitive violation of others' rights/ rules** – aggression to people/ animals, destruction of property, lying, theft, truancy, running away; may → antisocial PD	Parent management training, multisystemic therapy, limit setting
Oppositional defiant disorder	Angry, argumentative, defiant towards authority (no serious violation)	Parent training
Specific learning disorder	**Dyslexia (reading – most common)**, dysgraphia, dyscalculia; normal IQ	Remedial education
Tic disorders / Tourette's syndrome	**Motor + vocal tics > 1 yr**; **coprolalia** (obscene words); onset < 18	Haloperidol, risperidone, aripiprazole, clonidine; habit reversal
Enuresis	**≥ 5 yrs**	Alarm, star chart, desmopressin
Encopresis	**≥ 4 yrs**	Bowel training
Separation anxiety disorder	Excessive fear of separation from attachment figure; **school refusal**	CBT, gradual return to school
Selective mutism	Doesn't speak in specific situations (school)	–
Childhood depression	Irritability rather than sadness, school decline	–
Stuttering	Childhood-onset fluency disorder	Speech therapy

- **Child abuse & neglect** – recognise, document, report (**POCSO 2012, CHILDLINE 1098**).
- **Child Guidance Clinic** – multidisciplinary (child psychiatrist, psychologist, social worker, nurse).
- **Play therapy, family therapy, parent training** – key modalities.

10.5 Geriatric Psychiatry

- **Common disorders in elderly** – **depression (most common)**, **dementia**, **delirium**, anxiety, substance (BZD, alcohol), late-onset psychosis.
- **Elderly depression** – somatic complaints, **pseudodementia** (cognitive impairment due to depression – reversible; patient complains of memory loss – "don't know" answers), **↑ suicide risk (elderly males highest)**.
- **Principles** – **start low, go slow** (drugs), **avoid anticholinergics & long-acting BZD** (falls, confusion), polypharmacy review, social support, elder abuse screening.

✎ PYQ style

- Acute fluctuating confusion with impaired attention – **delirium**.
- Key difference delirium vs dementia – **consciousness/ attention impaired in delirium**.
- Sundowning – **worse in evening (dementia/ delirium)**.
- Delirium nursing – **well-lit room, re-orientation, familiar objects**.
- Most common dementia – **Alzheimer's**; second – **vascular**.
- Alzheimer's pathology – **amyloid plaques, neurofibrillary tangles, ↓ ACh**.
- 4 A's of Alzheimer's – **amnesia, aphasia, apraxia, agnosia**.
- Alzheimer's drug – **donepezil** (cholinesterase inhibitor); NMDA antagonist – **memantine**.
- Reversible dementia – **hypothyroidism, B12 deficiency, NPH**.
- NPH triad – **dementia, gait disturbance, urinary incontinence**.
- Depression mimicking dementia – **pseudodementia**.
- MMSE max – **30**.
- Past memories therapy – **reminiscence therapy**.
- Most common preventable cause of ID – **congenital hypothyroidism**; inherited – **Fragile X**.
- Autism onset – **< 3 yrs**; first described – **Kanner**.
- ADHD drug – **methylphenidate**.
- Tourette's – **motor + vocal tics, coprolalia**.
- Most common learning disorder – **dyslexia**.
- Most common psychiatric disorder in elderly – **depression**.

11 Psychiatric Emergencies, Community Mental Health & Legal Aspects

11.1 Psychiatric Emergencies

Emergency	Key management
Suicidal patient	**Highest priority – safety**, constant observation, remove harmful objects (see Ch 6)
Violent/ aggressive patient	See below
Catatonic stupor/ excitement	**IV/ IM lorazepam** (lorazepam challenge test), **ECT**; nutrition, fluids, prevent pressure sores/ DVT
Neuroleptic Malignant Syndrome	Stop antipsychotic, cooling, IV fluids, **dantrolene/ bromocriptine**, ICU
Serotonin syndrome	Stop serotonergic drugs, **cyproheptadine**, supportive
Lithium toxicity	Stop lithium, IV saline, **haemodialysis**
Acute dystonia	**IM promethazine/ benztropine/ diphenhydramine**
Delirium tremens	BZD, thiamine, fluids
Drug overdose	ABC, decontamination, antidotes (**naloxone – opioids, flumazenil – BZD**), TCA overdose – **sodium bicarbonate** (cardiac)
Panic attack	Stay with patient, calm, breathing exercises, BZD
Postpartum psychosis	Admission, safety of mother & baby
Acute psychosis/ excitement	**Rapid tranquillisation – IM haloperidol + promethazine** (or IM lorazepam/ olanzapine)
Conversion/ dissociative states	Calm reassurance, avoid secondary gain
Grief/ crisis	Crisis intervention

Management of violence & aggression

- **Predictors** – **history of violence (best predictor)**, substance intoxication, command hallucinations, paranoid delusions, mania, antisocial PD, young male; **warning signs – pacing, clenched fists, loud voice, threats, staring, agitation**.
- **Assault cycle** – **trigger → escalation → crisis → recovery → post-crisis depression**.
- **De-escalation (first-line)** – **remain calm, speak softly & slowly, non-threatening posture**, **maintain safe distance (> arm's length, ~1.5 m)**, **stand at an angle (not face-to-face)**, **keep exit route clear for both**, don't turn back, acknowledge feelings, **offer choices**, set clear limits, avoid arguing/ sudden movements, **reduce stimuli/ remove audience**, **never approach alone – call for help (team)**; remove potential weapons; avoid wearing necklaces/ stethoscope around neck.
- **If de-escalation fails** – **PRN medication** (oral first, then IM), **physical restraint by trained team (minimum 4–5 persons, one for each limb + head)**, as per MHCA (**last resort, documented, monitored every 15 min**).

- **Post-incident** – debriefing of patient & staff, documentation, review care plan.

11.2 Community Mental Health

- **Community psychiatry** – prevention, early detection, treatment & rehabilitation **in the community** rather than institutions; **deinstitutionalisation**.
- **Levels of prevention in mental health (Caplan)** – **primary** (prevent occurrence – mental health promotion, life skills education, stress management, genetic counselling, prenatal care, prevent child abuse), **secondary** (early detection & prompt treatment, crisis intervention, screening), **tertiary** (rehabilitation, prevent relapse & disability).
- **NMHP (1982)** – objectives: availability & accessibility of **minimum mental health care for all**, integration with general health care, community participation; **DMHP (1996 – pilot Bellary, Karnataka)** – district-level services, training of PHC staff, IEC, **suicide prevention, life skills, workplace stress, counselling in schools/ colleges**.
- **National Mental Health Policy (2014)**, **Mental Healthcare Act 2017**, **Tele-MANAS (10 Oct 2022) – 14416 / 1-800-891-4416** (24×7, multilingual), **KIRAN helpline (1800-599-0019, 2020)**, **Manodarpan** (students, 2020), **Ayushman Arogya Mandir – mental health as CPHC package**.
- **Rehabilitation services** – **day hospital/ day care centre, half-way home (transitional – between hospital & community), long-stay homes, sheltered workshop, vocational rehabilitation, supported employment, foster care, self-help groups, community-based rehabilitation**.
- **Stigma reduction**, **psychoeducation**, **family support (high EE reduction)**, **assertive community treatment**.
- **Role of community mental health nurse** – case finding, home visits, **medication management (depot injections)**, follow-up, crisis intervention, psychoeducation, liaison, counselling, referral, rehabilitation.

11.3 Mental Healthcare Act 2017 (MHCA)

- **Passed – 7 April 2017**; **came into force – 29 May 2018**; replaced **Mental Health Act 1987**; **rights-based**, in line with **UNCRPD**.

Section/ Provision	Key point
Sec 2 – Mental illness definition	Substantial disorder of thinking, mood, perception, orientation or memory that grossly impairs judgement, behaviour, capacity to recognise reality (**excludes mental retardation/ ID**)
Sec 4 – Capacity	Every person presumed to have capacity to make mental health care decisions
Sec 5 – Advance directive	Person can state **how they wish/ don't wish to be treated** & appoint nominated representative
Sec 14 – Nominated representative	Person appointed to support decision-making
Sec 18 – Right to access mental health care	Affordable, good quality, from govt services; free for BPL/ homeless
Sec 19 – Right to community living	–

Section/ Provision	Key point
Sec 20 – Right to protection from cruel, inhuman & degrading treatment	**No chaining**, clean environment, privacy, food, sanitation
Sec 21 – Equality & non-discrimination	**Medical insurance for mental illness on par with physical illness**
Sec 22 – Right to information, **Sec 23 – confidentiality**, **Sec 27 – free legal aid**	–
Sec 85–86 – Independent admission	**Voluntary** – person with capacity; can request discharge (**within 24 hrs**)
Sec 89 – Supported admission (up to 30 days)	High support needs – self-harm/ harm to others/ unable to care for self; **by nominated representative's request + 2 psychiatrists/ MHPs' assessment**
Sec 90 – Supported admission beyond 30 days	**Up to 90 days → 120 days → 180 days** with **Mental Health Review Board (MHRB)** approval
Sec 94 – Emergency treatment	**Up to 72 hours** (to prevent death/ harm) by registered medical practitioner
Sec 95 – Prohibited procedures	**ECT without anaesthesia & muscle relaxant**, **ECT for minors**, **sterilisation as treatment**, **chaining**
Sec 96 – Psychosurgery	Only with informed consent + MHRB approval
Sec 97 – Restraint & seclusion	Physical restraint only to prevent imminent harm; **seclusion & solitary confinement prohibited**
Sec 98 – Discharge planning	Community-based treatment
Sec 100 – Duty of police	Take person found wandering to public health establishment (within 24 hrs)
Sec 102 – Magistrate	Can order assessment
Sec 115 – Attempted suicide	**Presumed to have severe stress; not punishable; govt to provide care & rehabilitation**
Authorities	**Central Mental Health Authority (CMHA)**, **State Mental Health Authority (SMHA)**, **Mental Health Review Boards (MHRB)** – district level; register mental health establishments & professionals

11.4 Forensic Psychiatry & Legal Terms

- **McNaughten (M'Naghten) rule (1843)** – **legal insanity defence**: at the time of the act, due to unsoundness of mind, the person **did not know the nature of the act or that it was wrong**; India – **IPC Section 84 → BNS 2023 Section 22** ("act of a person of unsound mind").
- **Testamentary capacity** – capacity to make a valid **will** (understand nature, extent of property, claims of relatives); **lucid interval** wills valid.
- **Competency to stand trial**, **criminal responsibility**, **civil responsibility (contracts, marriage)** – mentally ill person's contract voidable.
- **Marriage** – **Hindu Marriage Act 1955** – mental disorder (unfit for marriage/ procreation) ground for **annulment/ divorce**.

- **Narcoanalysis** – IV thiopentone/ sodium amytal (truth serum) – **requires consent** (SC 2010, Selvi case).
- **Rights of Persons with Disabilities Act 2016** – mental illness one of 21 disabilities.
- **NDPS Act 1985**, **POCSO 2012**, **Juvenile Justice Act 2015**.

11.5 Ethics in Psychiatric Nursing

- **Autonomy** (informed consent, refusal), **beneficence**, **non-maleficence**, **justice**, **confidentiality** (exceptions – **duty to warn/ protect third party – Tarasoff case**, court order, risk of harm), **veracity**, **fidelity**, **least restrictive environment**, **dignity**, **advocacy**.
- **Informed consent** – capacity, information, voluntariness.

PYQ style

- Best predictor of violence – **past history of violence**.
- First step in managing aggression – **verbal de-escalation**.
- Safe distance from aggressive patient – **more than arm's length; stand at an angle; exit clear**.
- Restraint – **trained team (≥ 4–5), last resort, check every 15 min**.
- Rapid tranquillisation – **IM haloperidol + promethazine / lorazepam**.
- Catatonia Rx – **lorazepam, ECT**.
- TCA overdose cardiac toxicity – **sodium bicarbonate**.
- Half-way home – **transitional between hospital & community**.
- DMHP started – **1996, Bellary**.
- MHCA 2017 came into force – **29 May 2018**.
- Advance directive – **Sec 5**; nominated representative – **Sec 14**.
- Supported admission – **up to 30 days (Sec 89)**; beyond – **Sec 90 (MHRB)**.
- Emergency treatment – **72 hours (Sec 94)**.
- ECT in minors & without anaesthesia – **prohibited (Sec 95)**.
- Chaining, seclusion – **prohibited**.
- Attempted suicide – **Sec 115 (not punishable)**.
- District-level review body – **Mental Health Review Board**.
- Insanity defence – **McNaughten rule; BNS Sec 22 (old IPC 84)**.
- Capacity to make a will – **testamentary capacity**.
- Duty to warn third party – **Tarasoff**.

12 Quick Revision – Master Tables & 150 One-Liners

12.1 Master Table – Durations & Numbers

Item	Value
Depression	**≥ 2 weeks**
Mania / hypomania	**≥ 1 week / ≥ 4 days**
Dysthymia / cyclothymia	**≥ 2 years**
Schizophrenia (ICD / DSM-5)	**≥ 1 month / ≥ 6 months**
GAD	**≥ 6 months**
PTSD	**> 1 month**
Acute stress disorder	**< 1 month**
Rapid cycling	**≥ 4 episodes/ year**
Antidepressant onset	**2–4 weeks**
Lithium therapeutic / toxic	**0.6–1.2 / > 1.5 mEq/L**
Lithium sample	**12 hrs after last dose**
Alcohol tremors / seizures / DT	**6–12 h / 12–48 h / 48–72 h**
ECT NPO	**6–8 hrs**
ECT sessions	**6–12, 2–3/week**
Adequate seizure	**≥ 20–25 sec**
MMSE	**30 (< 24 impaired)**
Intellectual disability	**IQ < 70** (mild 50–69, moderate 35–49, severe 20–34, profound < 20)
Enuresis / encopresis	**≥ 5 yrs / ≥ 4 yrs**
Anorexia BMI	**< 17.5**
Crisis resolution	**4–6 weeks**
Emergency treatment (MHCA)	**72 hrs**
Supported admission (MHCA)	**≤ 30 days (Sec 89)**
NMHP / DMHP	**1982 / 1996**
MHCA passed / in force	**2017 / 29 May 2018**
Tele-MANAS	**14416**
World Mental Health Day	**10 October**
World Suicide Prevention Day	**10 September**
World Alzheimer's Day	**21 September**
Anti-drug day	**26 June**

12.2 Master Table – Drugs

Drug	Use / key fact
Chlorpromazine	First antipsychotic; photosensitivity
Haloperidol	High potency; EPS; acute agitation
Clozapine	Resistant schizophrenia; **agranulocytosis**
Risperidone	Hyperprolactinaemia
Olanzapine	Weight gain, diabetes
Fluphenazine decanoate	Depot
Trihexyphenidyl	Parkinsonism (EPS)
Promethazine IM	Acute dystonia
Propranolol	Akathisia
Dantrolene / bromocriptine	NMS
Fluoxetine	SSRI, longest half-life
Imipramine	First TCA; enuresis
MAOI	Cheese reaction
Cyproheptadine	Serotonin syndrome
Lithium	Bipolar prophylaxis; Ebstein's anomaly
Valproate	Rapid cycling; hepatotoxic, NTD
Carbamazepine	Agranulocytosis, SJS
Lorazepam	Catatonia; liver disease detox
Chlordiazepoxide	Alcohol withdrawal
Flumazenil	BZD antidote
Buspirone	Non-addictive anxiolytic
Disulfiram	Alcohol aversion
Naltrexone / acamprosate	Anti-craving
Naloxone	Opioid overdose
Buprenorphine / methadone	OST
Donepezil / memantine	Alzheimer's
Methylphenidate	ADHD
Modafinil	Narcolepsy
Thiamine	Wernicke's prevention

12.3 150 One-Liners

One-liners 1–50

1. Removed chains – **Pinel**.
2. Schizophrenia term – **Bleuler**.
3. Dementia praecox – **Kraepelin**.
4. First-rank symptoms – **Schneider**.
5. Psychoanalysis – **Freud**.
6. Mother of psychiatric nursing – **Peplau**.
7. First psychiatric nurse (USA) – **Linda Richards**.
8. ECT – **Cerletti & Bini 1938**.
9. Lithium – **John Cade**.
10. Lobotomy – **Egas Moniz**.
11. ICD – **WHO**; DSM – **APA**.
12. Hallucination – **perception without stimulus**.
13. Illusion – **misinterpretation of real stimulus**.
14. Delusion – **fixed false belief**.
15. Commonest delusion – **persecutory**.
16. Commonest hallucination (schizophrenia) – **auditory**.
17. Visual hallucination – **organic/ delirium**.
18. Formication – **cocaine**.
19. Flight of ideas – **mania**.
20. Loosening of association – **schizophrenia**.
21. Clang association – **mania**.
22. Neologism – **schizophrenia**.
23. Confabulation – **Korsakoff**.
24. Perseveration – **organic**.
25. Waxy flexibility – **catatonia**.
26. Echolalia – **catatonia/ autism**.
27. Mood – **subjective**; affect – **objective**.
28. Anhedonia – **loss of pleasure**.
29. Orientation lost first – **time**.
30. Id – **pleasure principle**.
31. Ego – **reality principle**.
32. Superego – **conscience**.
33. Conscious defence – **suppression**.
34. Mature defence – **sublimation**.
35. Alcoholic's defence – **denial**.
36. Blaming others – **projection**.
37. Opposite behaviour – **reaction formation**.
38. Childish behaviour – **regression**.
39. Excuse – **rationalisation**.
40. Anger shifted – **displacement**.

41. OCD defence – **undoing**.
42. MSE cognition scale – **MMSE**.
43. Abstract thinking – **proverbs**.
44. Concentration – **serial 7s**.
45. Projective test – **Rorschach**.
46. Johari window – **self-awareness**.
47. Transference – **patient → nurse feelings**.
48. Non-therapeutic – **false reassurance, "why"**.
49. Peplau phases – **orientation, identification, exploitation, resolution**.
50. Trust & contract – **orientation phase**.

One-liners 51–100

51. D2 blockade – **antipsychotics**.
52. Agranulocytosis – **clozapine**.
53. Irreversible EPS – **tardive dyskinesia**.
54. TD scale – **AIMS**.
55. Acute dystonia – **promethazine IM**.
56. Akathisia – **propranolol**.
57. NMS – **fever + rigidity + ↑ CPK**.
58. NMS drug – **dantrolene**.
59. First-line antidepressant – **SSRI**.
60. Lethal overdose – **TCA**.
61. Cheese reaction – **MAOI**.
62. Antidepressant suicide risk – **early recovery**.
63. Lithium level – **0.6–1.2**.
64. Lithium toxicity – **coarse tremor, vomiting, diarrhoea**.
65. Lithium & salt – **keep normal**.
66. Lithium hypothyroidism – **yes**.
67. BZD antidote – **flumazenil**.
68. ECT main indication – **suicidal depression**.
69. ECT contraindication – **raised ICP**.
70. ECT relaxant – **succinylcholine**.
71. ECT side effect – **memory loss**.
72. CBT – **Aaron Beck**.
73. Phobia therapy – **systematic desensitisation (Wolpe)**.
74. OCD therapy – **ERP**.
75. Alcohol aversion – **aversion therapy/ disulfiram**.
76. Token economy – **operant conditioning**.
77. Therapeutic community – **Maxwell Jones**.
78. Crisis intervention – **Caplan, 4–6 weeks**.
79. Psychodrama – **Moreno**.
80. Schizophrenia prevalence – **1 %**.
81. Dopamine excess – **schizophrenia**.
82. Bleuler's 4 A's – **association, affect, ambivalence, autism**.
83. Commonest type – **paranoid**.
84. Worst prognosis – **hebephrenic**.
85. Best prognosis – **catatonic**.
86. Negative symptoms – **poor prognosis**.
87. High EE – **relapse**.
88. Hallucinating patient – **present reality**.
89. Delusions – **don't argue**.
90. Folie à deux – **shared psychosis**.
91. Cotard – **nihilistic delusion**.
92. Capgras – **impostor**.

93. Othello – **jealousy**.
94. Depression sleep – **early morning awakening**.
95. Depression – **worse in morning**.
96. Mania – **DIG FAST**.
97. Manic activity – **non-competitive gross motor**.
98. Manic diet – **finger foods**.
99. Bipolar prophylaxis – **lithium**.
100. Suicide strongest predictor – **previous attempt**.

One-liners 101–150

101. Suicide method India – **hanging**.
102. Suicide scale – **SAD PERSONS**.
103. Suicide precaution – **constant observation**.
104. Asking about suicide – **safe**.
105. Sec 115 MHCA – **suicide attempt decriminalised**.
106. GAD – **≥ 6 months**.
107. Panic – **sudden intense fear**.
108. Agoraphobia – **open places/ crowds**.
109. Acrophobia – **heights**.
110. Claustrophobia – **closed spaces**.
111. Commonest compulsion – **washing**.
112. OCD drug – **SSRI/ clomipramine**.
113. PTSD – **flashbacks > 1 month**.
114. GAS – **Selye**.
115. Grief stages – **Kübler-Ross DABDA**.
116. La belle indifférence – **conversion disorder**.
117. Fugue – **travel + amnesia**.
118. Hypochondriasis – **fear of disease**.
119. Munchausen – **factitious**.
120. Malingering – **external gain**.
121. Dependence – **craving, tolerance, withdrawal**.
122. Commonest substance – **alcohol (after tobacco)**.
123. CAGE – **≥ 2**.
124. GGT – **alcohol marker**.
125. DT – **48–72 hrs, visual hallucinations**.
126. Detox – **chlordiazepoxide**.
127. Thiamine – **before glucose**.
128. Wernicke – **confusion, ataxia, ophthalmoplegia**.
129. Korsakoff – **amnesia + confabulation**.
130. Disulfiram – **aldehyde dehydrogenase**.
131. AA – **12 steps**.
132. Opioid pupils – **pin-point**.
133. Opioid withdrawal pupils – **dilated**.
134. Cannabis – **amotivational syndrome**.
135. NDPS Act – **1985**.
136. Antisocial PD – **no guilt**.
137. Borderline PD – **self-harm, splitting**.
138. Borderline therapy – **DBT**.
139. Histrionic – **attention seeking**.
140. Anorexia – **highest mortality**.
141. Bulimia – **Russell's sign**.
142. Narcolepsy – **cataplexy**.

143. Delirium – **acute, fluctuating, attention ↓**.
144. Commonest dementia – **Alzheimer's**.
145. Alzheimer's – **↓ acetylcholine**.
146. NPH – **wet, wobbly, wacky**.
147. Autism – **< 3 yrs, poor eye contact**.
148. ADHD – **methylphenidate**.
149. Violence predictor – **past violence**.
150. Insanity defence – **McNaughten / BNS 22**.



Exam strategy – Asha Didi ki last advice

- GSSSB paper me **Mental Health se ~8–12 questions** aate hain – **psychiatric terms (hallucination, delusion, flight of ideas), defence mechanisms, drugs & side effects (lithium, EPS, NMS), ECT care, schizophrenia, depression/ suicide, alcohol withdrawal aur MHCA 2017**.
- **Therapeutic communication** wale questions me hamesha **patient ke feelings par focus** wala answer chuno; "false reassurance", "why" aur "arguing" galat options hote hain.
- Safety pehle – **suicide/ violence** wale scenario me "safety/ observation" answer priority hai.
- MCQ Bank PDF ke saare questions **2 baar** solve karo, galtiyan notebook me likho.

— Mental Health Nursing · GSSSB Staff Nurse Preparation · Padhte raho, revise karte raho! —